# Answer Key & Rationales · Form 3

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.23-0423.

| # | Key | Area | Sub-area | P |
|---|---|---|---|---|
| 1 | **A** | III | III.C | |
| 2 | **C** | III | III.B | |
| 3 | **B** | II | II.A | |
| 4 | **A** | I | I.B | |
| 5 | **C** | I | I.A | |
| 6 | **C** | I | I.C | |
| 7 | **A** | III | III.B | **P** |
| 8 | **B** | I | I.B | |
| 9 | **B** | I | I.B | |
| 10 | **C** | I | I.B | |
| 11 | **A** | III | III.B | |
| 12 | **B** | II | II.C | |
| 13 | **B** | III | III.D | |
| 14 | **B** | II | II.B | |
| 15 | **D** | III | III.B | |
| 16 | **B** | II | II.C | |
| 17 | **B** | I | I.A | **P** |
| 18 | **A** | I | I.A | |
| 19 | **B** | II | II.A | |
| 20 | **C** | I | I.B | |
| 21 | **B** | II | II.B | |
| 22 | **B** | III | III.B | |
| 23 | **A** | I | I.C | |
| 24 | **B** | III | III.C | |
| 25 | **D** | III | III.A | |
| 26 | **A** | I | I.C | |
| 27 | **C** | II | II.A | |
| 28 | **B** | II | II.A | **P** |
| 29 | **B** | I | I.A | |
| 30 | **C** | III | III.B | |
| 31 | **C** | II | II.A | |
| 32 | **A** | III | III.A | |
| 33 | **B** | II | II.A | |
| 34 | **A** | II | II.A | **P** |
| 35 | **B** | III | III.B | |
| 36 | **B** | III | III.B | |
| 37 | **B** | II | II.B | |
| 38 | **A** | III | III.B | |
| 39 | **C** | III | III.B | **P** |
| 40 | **B** | I | I.A | |
| 41 | **A** | I | I.B | |
| 42 | **C** | I | I.C | |
| 43 | **C** | I | I.B | |
| 44 | **C** | III | III.A | |
| 45 | **B** | III | III.B | |
| 46 | **C** | II | II.A | |
| 47 | **D** | I | I.A | |
| 48 | **C** | I | I.A | |
| 49 | **B** | I | I.B | **P** |
| 50 | **D** | II | II.C | |
| 51 | **A** | II | II.A | |
| 52 | **C** | III | III.D | |
| 53 | **D** | I | I.C | |
| 54 | **B** | I | I.B | |
| 55 | **A** | II | II.B | |
| 56 | **B** | III | III.B | |
| 57 | **A** | II | II.A | |
| 58 | **C** | I | I.B | |
| 59 | **B** | II | II.B | |
| 60 | **D** | II | II.A | |
| 61 | **C** | II | II.B | |
| 62 | **A** | I | I.A | **P** |
| 63 | **B** | II | II.A | |
| 64 | **C** | II | II.B | |
| 65 | **B** | I | I.B | |
| 66 | **C** | III | III.B | |
| 67 | **A** | I | I.A | **P** |
| 68 | **C** | III | III.B | |
| 69 | **B** | III | III.B | |
| 70 | **A** | III | III.D | |
| 71 | **A** | III | III.C | |
| 72 | **A** | II | II.A | |
| 73 | **B** | III | III.D | |
| 74 | **C** | I | I.B | |
| 75 | **C** | I | I.C | |
| 76 | **C** | II | II.A | |
| 77 | **D** | III | III.C | |
| 78 | **C** | III | III.B | |
| 79 | **C** | I | I.A | |
| 80 | **A** | I | I.C | |
| 81 | **D** | I | I.A | |
| 82 | **A** | III | III.B | |
| 83 | **B** | III | III.A | |
| 84 | **A** | II | II.B | |
| 85 | **C** | I | I.B | |
| 86 | **B** | I | I.B | |
| 87 | **C** | II | II.A | |
| 88 | **B** | III | III.B | |
| 89 | **B** | II | II.A | |
| 90 | **A** | III | III.B | |
| 91 | **A** | II | II.C | |
| 92 | **C** | II | II.B | |
| 93 | **C** | I | I.A | |
| 94 | **C** | III | III.A | |
| 95 | **B** | II | II.A | |
| 96 | **C** | I | I.B | |
| 97 | **A** | I | I.C | |
| 98 | **B** | III | III.C | |
| 99 | **A** | I | I.B | |
| 100 | **C** | II | II.B | |
| 101 | **C** | II | II.B | |
| 102 | **D** | III | III.B | |
| 103 | **A** | I | I.C | |
| 104 | **A** | I | I.B | |
| 105 | **A** | I | I.A | |
| 106 | **B** | I | I.A | |
| 107 | **C** | I | I.B | |
| 108 | **A** | II | II.A | |
| 109 | **A** | III | III.A | **P** |
| 110 | **B** | II | II.A | |
| 111 | **D** | I | I.A | |
| 112 | **B** | III | III.B | **P** |
| 113 | **D** | I | I.C | |
| 114 | **D** | III | III.B | |
| 115 | **B** | I | I.C | |
| 116 | **C** | II | II.C | |
| 117 | **C** | II | II.A | |
| 118 | **A** | II | II.B | |
| 119 | **C** | III | III.A | |
| 120 | **A** | II | II.A | **P** |
| 121 | **B** | II | II.A | **P** |
| 122 | **D** | III | III.B | |

## Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

| Area / sub-area | Scored item numbers | Items | Correct |
|---|---|---|---|
| **I. Values and Ethics** | | **40** | **____ / 40** |
| I.A Ethical Principles and Responsibilities | 5, 18, 29, 40, 47, 48, 79, 81, 93, 105, 106, 111 | 12 | ____ |
| I.B Ethical Service Delivery | 4, 8, 9, 10, 20, 41, 43, 54, 58, 65, 74, 85, 86, 96, 99, 104, 107 | 17 | ____ |
| I.C Diversity and Social Justice | 6, 23, 26, 42, 53, 75, 80, 97, 103, 113, 115 | 11 | ____ |
| **II. Assessment and Planning** | | **35** | **____ / 35** |
| II.A Assessment Concepts | 3, 19, 27, 31, 33, 46, 51, 57, 60, 63, 72, 76, 87, 89, 95, 108, 110, 117 | 18 | ____ |
| II.B Assessment Methods and Techniques | 14, 21, 37, 55, 59, 61, 64, 84, 92, 100, 101, 118 | 12 | ____ |
| II.C Assessment Practices | 12, 16, 50, 91, 116 | 5 | ____ |
| **III. Intervention and Practice** | | **35** | **____ / 35** |
| III.A Practice Concepts | 25, 32, 44, 83, 94, 119 | 6 | ____ |
| III.B Intervention Methods and Techniques | 2, 11, 15, 22, 30, 35, 36, 38, 45, 56, 66, 68, 69, 78, 82, 88, 90, 102, 114, 122 | 20 | ____ |
| III.C Practice Evaluation and Research | 1, 24, 71, 77, 98 | 5 | ____ |
| III.D Supervision and Administration | 13, 52, 70, 73 | 4 | ____ |

**Pretest items (skip when scoring):** 7, 17, 28, 34, 39, 49, 62, 67, 109, 112, 120, 121

| | |
|---|---|
| **Total correct (scored items)** | _____ / 110 |
| **Practice pass line** | 77 / 110  (70%) |
| **Correct needed to reach the line** | 77 − your total = _____ |

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

## Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

| Raw | % | Raw | % | Raw | % | Raw | % | Raw | % | Raw | % |
|---|---|---|---|---|---|---|---|---|---|---|---|
| 110 | 100% | 91 | 83% | 72 | 65% | 53 | 48% | 34 | 31% | 15 | 14% |
| 109 | 99% | 90 | 82% | 71 | 65% | 52 | 47% | 33 | 30% | 14 | 13% |
| 108 | 98% | 89 | 81% | 70 | 64% | 51 | 46% | 32 | 29% | 13 | 12% |
| 107 | 97% | 88 | 80% | 69 | 63% | 50 | 45% | 31 | 28% | 12 | 11% |
| 106 | 96% | 87 | 79% | 68 | 62% | 49 | 45% | 30 | 27% | 11 | 10% |
| 105 | 95% | 86 | 78% | 67 | 61% | 48 | 44% | 29 | 26% | 10 | 9% |
| 104 | 95% | 85 | 77% | 66 | 60% | 47 | 43% | 28 | 25% | 9 | 8% |
| 103 | 94% | 84 | 76% | 65 | 59% | 46 | 42% | 27 | 25% | 8 | 7% |
| 102 | 93% | 83 | 75% | 64 | 58% | 45 | 41% | 26 | 24% | 7 | 6% |
| 101 | 92% | 82 | 75% | 63 | 57% | 44 | 40% | 25 | 23% | 6 | 5% |
| 100 | 91% | 81 | 74% | 62 | 56% | 43 | 39% | 24 | 22% | 5 | 5% |
| 99 | 90% | 80 | 73% | 61 | 55% | 42 | 38% | 23 | 21% | 4 | 4% |
| 98 | 89% | 79 | 72% | 60 | 55% | 41 | 37% | 22 | 20% | 3 | 3% |
| 97 | 88% | 78 | 71% | 59 | 54% | 40 | 36% | 21 | 19% | 2 | 2% |
| 96 | 87% | **77** | **70%** | 58 | 53% | 39 | 35% | 20 | 18% | 1 | 1% |
| 95 | 86% | 76 | 69% | 57 | 52% | 38 | 35% | 19 | 17% | 0 | 0% |
| 94 | 85% | 75 | 68% | 56 | 51% | 37 | 34% | 18 | 16% | | |
| 93 | 85% | 74 | 67% | 55 | 50% | 36 | 33% | 17 | 15% | | |
| 92 | 84% | 73 | 66% | 54 | 49% | 35 | 32% | 16 | 15% | | |

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

# Rationales for every item

## Section 1

### 1. INT-1667 (rev 2) · Best answer: A

A new scale is given to a group. Its items correlate strongly with one another. Which property does this result support?

**A.** Internal consistency reliability
**B.** Test-retest reliability
**C.** Construct validity

**Why each option is right or wrong**

**A** **Correct.** Internal consistency reliability asks whether a scale's items measure the same thing, judged by how strongly they correlate with one another on a single administration, usually summarized with Cronbach's alpha (Bloom, Fischer & Orme, 2009). It beats C because items that hang together show the scale is reliable, not that it measures the intended concept; validity needs other evidence, such as expected relationships with other measures, and a scale can be reliable without being valid.

**B** **Incorrect** (trap: Wrong model or concept). Test-retest reliability correlates scores from two administrations of the same scale to show that scores stay stable over time. Correlations among the items describe how the items relate to each other, not stability across time.

**C** **Incorrect** (trap: Wrong model or concept). Construct validity asks whether a scale measures the intended concept, shown by expected relationships with other measures. Strong item correlations show reliability, which is necessary but not enough for validity.

**Takeaway:** Strong item intercorrelation on one administration shows internal consistency; it supports reliability, not validity.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, reliability: internal consistency and test-retest
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 2. INT-1536 (rev 1) · Best answer: C

A client with schizophrenia has been hospitalized 4 times in the past year and is staying in a shelter. The client misses most monthly office visits with an outpatient case manager and stops medication between admissions. The client says they want to stay out of the hospital. What is the **MOST** appropriate step for the case manager?

**A.** Increase to weekly office visits with a reminder call before each
**B.** Ask the psychiatrist to consider a long-acting injectable medication
**C.** Refer the client to an assertive community treatment team

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). More frequent office visits keep the same office-based model the client has not used. This pattern calls for outreach where the client lives. Weekly visits fit a client who attends but needs more support.

**B** **Incorrect** (trap: Good but not best). Raising medication options with the prescriber is reasonable, but it addresses only adherence, and the choice belongs to the prescriber. Housing instability and missed visits would remain. The question can be revisited within the ACT team's work.

**C** **Correct.** Assertive community treatment is designed for adults with serious mental illness who cycle through hospitals, face unstable housing and are not engaged by office-based services (SAMHSA ACT EBP KIT, 2008). A multidisciplinary team shares the caseload, delivers most services where the client lives and offers round-the-clock coverage. Four admissions, shelter living and missed visits meet these indications, and the client's goal of staying out of the hospital fits. It beats B because a medication change addresses one piece; ACT brings medication support, housing help and outreach together.

**Takeaway:** Adults with serious mental illness who cycle through hospitals and miss office-based care need assertive community treatment's team-based outreach.

**Sources:** SAMHSA (2008) Assertive Community Treatment (ACT) Evidence-Based Practices KIT, Getting Started with Evidence-Based Practices: who benefits from ACT
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 3. ASM-1643 (rev 2) · Best answer: B

During a home visit, a parent says their 4-year-old pointed at a person with darker skin in a store and asked loudly why the person's skin was 'dirty.' The embarrassed parent hushed the child. The parent says that race was never discussed at home growing up and now plans to avoid the topic so the child will not 'notice differences.' What is the **MOST** appropriate response by the social worker?

**A.** Agree to wait on race talk until the child is older
**B.** Say that preschoolers register race and gain from open talk
**C.** Explore further how the parent's family handled talk about race

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Many adults believe young children are too young to notice race, but this child's question shows the opposite. Waiting until the child is older does not prevent noticing; it leaves the child to form ideas without guidance.

**B** **Correct.** Children notice racial differences early, and by preschool many have begun forming racial attitudes. Research links open, age-appropriate conversation about race with lower bias in young children, while silence leaves them to draw their own conclusions. This child has already noticed, so the parent's plan rests on a misconception; correcting it gently and encouraging simple, honest answers supports healthy development. It beats C because the parent has already described the family history behind the plan and now needs accurate information (Katz, 2003).

**C** **Incorrect** (trap: Good but not best). Exploring a parent's background helps when the source of discomfort is unclear. This parent has already described a family that never

discussed race, so more exploration delays correcting the belief that silence keeps the child from noticing.

**Takeaway:** Young children notice race early; open, age-appropriate talk about differences supports healthy racial attitudes better than silence.

**Sources:** Katz, P. A. (2003). Racists or tolerant multiculturalists? How do they begin? American Psychologist, 58(11), 897-909, whole article

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.19 Racial, ethnic, cultural, spiritual development across the lifespan · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 4. ETH-0113 (rev 2) · Best answer: A

A social worker sees an adult client with generalized anxiety disorder by video each week. The client will spend the next month at a relative's home in another state. The social worker confirms that this state's rules permit the sessions to continue. What should the social worker do **NEXT**?

**A.** Identify emergency services near where the client will stay

**B.** Continue weekly sessions as usual under the current safety plan

**C.** Ask the client to pause sessions until returning home

#### Why each option is right or wrong

**A** **Correct.** Telehealth practice takes place where the client is located, and social workers should know the emergency resources in that location (Tech Standard 2.13). Legal authority is settled, so the next step is updating the emergency plan with services near the relative's home, along with the client's address there. It beats B because the current plan lists resources in the home state that cannot help the client during the stay. Social workers follow the law where the client is located (NASW Code 1.04(e); Tech Standard 2.02).

**B** **Incorrect** (trap: Ignores risk). The existing safety plan points to emergency resources in the home state. If a crisis occurs during the stay, those contacts cannot respond, so continuing unchanged leaves a gap in safety planning.

**C** **Incorrect** (trap: Hands off too soon). Pausing treatment for a month interrupts care the client needs when the social worker has already confirmed the sessions are allowed. A pause would fit only if the other state did not permit practice.

**Takeaway:** For telehealth with a client in a new location, confirm legal authority, then update the emergency plan for that location.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.13; NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.02; NASW Code of Ethics (2021), 1.04(e)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 5. ETH-1021 (rev 1) · Best answer: C

A resident of a skilled nursing facility has major neurocognitive disorder and has been found to lack capacity to make health care decisions. The resident's adult child, the legally appointed health care agent, has consented to the resident joining a weekly reminiscence group. What should the social worker do **NEXT**?

**A.** Enroll the resident now, since the agent has already consented
**B.** Have the resident sign the group consent form too
**C.** Explain the group simply and ask the resident's assent
**D.** Postpone the group until capacity improves

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The agent's consent authorizes participation, so enrolling is the step that follows. Doing it before explaining the group to the resident skips the Code's duty to inform the client and seek their agreement.

**B** **Incorrect** (trap: Misuses the technique). A person found to lack decision-making capacity cannot give valid legal consent, so a signature adds nothing legally. The resident's assent is sought through a simple explanation, not a form.

**C** **Correct.** When a client lacks capacity, the social worker seeks permission from an appropriate third party and also informs the client in a way the client can understand, acting consistently with the client's wishes and interests (NASW Code 1.03(c); NASW Code 1.14). With the agent's consent obtained, the next step is to explain the group simply and invite the resident's agreement. C beats A because the agent's consent makes participation lawful, but enrolling without involving the resident skips the duty to inform and seek assent.

**D** **Incorrect** (trap: Good but not best). Major neurocognitive disorder is usually progressive, so waiting for capacity to return could deny the resident a helpful activity indefinitely. Third-party consent plus the resident's assent allows participation now.

**Takeaway:** When a client lacks capacity, get consent from the authorized third party and still explain services to the client and seek assent.

**Sources:** NASW Code of Ethics (2021), 1.03(c); NASW Code of Ethics (2021), 1.14
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 6. ETH-1616 (rev 2) · Best answer: C

During a child welfare meeting with a parent who speaks Haitian Creole, the social worker notices that the agency interpreter is giving long replies the parent did not say. The interpreter then adds, in English, 'I told the parent to be grateful for the help.' What should the social worker do **FIRST**?

**A.** Ask a bilingual coworker to take over interpreting now
**B.** Report the interpreter to the language service afterward
**C.** Pause and restate the interpreter's role right away

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Acting now is right, which makes this tempting. A bilingual coworker who is not a qualified interpreter can add new errors and role confusion in a high-stakes meeting. If the agency interpreter cannot keep to the role once it is restated, a qualified replacement is the next step.

**B** **Incorrect** (trap: Right action, wrong time). Reporting the conduct may be appropriate after the meeting, especially if it continues. As a first step it lets the current meeting go on with inaccurate interpreting in a case with high stakes for the family.

**C** **Correct.** Interpreters should convey messages accurately and completely and should not counsel, advise or insert their own views (NCIHC, 2004). The interpreter is adding content and advising the parent in a meeting with high stakes, so the social worker should stop at once and restate the role: interpret everything said, add nothing, leave nothing out. It beats B because a report may follow, but waiting lets the meeting continue on distorted communication that could affect the parent's case and understanding (NASW Code 1.03(b)).

**Takeaway:** If an interpreter adds, omits or advises, stop and restate the interpreter's role at once; report the problem afterward if needed.

**Sources:** National Council on Interpreting in Health Care (2004). A National Code of Ethics for Interpreters in Health Care, ethical principles: accuracy; impartiality; NASW Code of Ethics (2021), 1.03(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 7. INT-1111 (rev 2) · Best answer: A

**PRETEST** — unscored on this form; do not count it

At an adult day program, a 79-year-old client says a new sleep medicine 'is working fine.' During a 30-minute meeting with the social worker, the client's head drops forward twice and the client loses track of questions. What should the social worker do **FIRST**?

**A.** Share what was observed and ask about daytime drowsiness
**B.** Contact the prescriber about a possible medication side effect
**C.** Accept the client's report and continue the meeting
**D.** Suggest the client take the medicine earlier in the evening

**Why each option is right or wrong**

**A** **Correct.** Observation is part of active listening: what the social worker sees can add to or contradict what the client says. The report that the medicine 'is working fine' conflicts with visible drowsiness and lost focus. Naming the observation tentatively and asking about daytime sleepiness lets the client explain and gathers details the prescriber will need, such as timing and any falls (Hepworth et al.). It beats B because contacting the prescriber is likely the next step, but doing it first skips the client's own account and their agreement to share it.

**B** **Incorrect** (trap: Right action, wrong time). Possible medication side effects in an older adult should reach the prescriber, and this will likely follow. First, the social worker checks the observation with the client, gathers details, and seeks the client's agreement to share them.

**C** **Incorrect** (trap: Ignores risk). Taking the report at face value ignores direct evidence of drowsiness in an older adult, which can raise the risk of falls and confusion. Observations that contradict self-report call for a gentle follow-up question.

**D** **Incorrect** (trap: Outside the social work role). Changing when a medication is taken is a prescribing decision. Social workers bring concerns to the prescriber; they do not adjust medication timing or doses, however well meant.

**Takeaway:** When observed behavior contradicts a client's report, share the observation tentatively and ask about it before acting on it.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: sources of information, including direct observation of nonverbal behavior
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 8. ETH-1516 (rev 3) · Best answer: B

On the day of discharge from an inpatient psychiatric unit, a patient thanks the social worker and asks for the worker's personal cell phone number, saying they fear no one will answer at night 'if things get bad again.' The patient starts outpatient care next week. What is the social worker's **BEST** response?

**A.** Share the number for after-hours emergencies
**B.** Provide clinic and 24-hour crisis contacts
**C.** Record the request in the discharge note
**D.** Explain the policy against sharing personal numbers

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Sharing the number feels caring, but it creates a personal channel the worker cannot staff around the clock. In a real crisis the patient might wait for a reply instead of reaching help that is always available.

**B** **Correct.** Giving a client a personal phone number blurs the professional boundary and invites contact the worker cannot reliably answer (NASW Code 1.06(c); Technology Standard 2.11). The patient's stated fear is that no one will answer at night, so the best response gives dependable professional options that answer at any hour: the outpatient clinic's line and 24-hour crisis contacts such as the 988 Suicide and Crisis Lifeline. Offering them also declines the personal number without a lecture. It beats D because explaining the policy answers the request but leaves the patient's fear of a night-time crisis unmet on the day of discharge.

**C** **Incorrect** (trap: Good but not best). Documenting the request and the response is good practice, but a note is not a response to the patient. It leaves the patient without a crisis plan at the moment of discharge.

**D** **Incorrect** (trap: Good but not best). Holding the boundary is correct, and a brief explanation can accompany the crisis contacts. Explaining the policy alone, though, answers the request while ignoring the need the patient named, help at night if things get bad, at the moment the patient is leaving the unit.

**Takeaway:** Meet a client's need for crisis support through professional, always-available channels rather than a worker's personal phone.

**Sources:** NASW Code of Ethics (2021), 1.06(c); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.11
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 9. ETH-1563 (rev 2) · Best answer: B

A senior center's counseling program for older adults will lose its grant in 60 days. The board has appealed the funder's decision, and a ruling is expected in about 6 weeks. Program staff worry that telling clients now will cause some to drop out early. What is the **BEST** way for the social worker to proceed with current clients?

**A.** Wait for the appeal ruling before saying anything to clients
**B.** Tell clients now and begin planning options with each one
**C.** Give every client a list of community agencies at the last session

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Waiting avoids alarming clients if the grant is restored, but it gambles their continuity on the appeal. If the ruling is negative, about 2 weeks would remain for transfers, which is not prompt notice.

**B** **Correct.** Social workers who anticipate the termination or interruption of services notify clients promptly and seek transfer, referral or continuation based on each client's needs and preferences (NASW Code 1.17(e)). With 60 days left, telling clients now, including that an appeal is pending, gives each one time to plan. Early notice also protects continuity of care for clients who will need time to find and reach new providers (1.15). It beats A because a ruling 2 weeks before closure would leave too little time for orderly transfers if the appeal fails; honest notice about uncertainty beats a late surprise.

**C** **Incorrect** (trap: Misuses the technique). A generic list handed out at the final session is a late, one-size referral that ignores each client's needs and preferences. Transfer planning should be individual and start well before services end.

**Takeaway:** When a program ending is anticipated, give prompt, honest notice and plan transitions individually, even while the outcome is uncertain.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 10. ETH-1513 (rev 1) · Best answer: C

A social worker in private practice is asked by a close friend to provide therapy for the friend's 15-year-old, who has been anxious and refusing school. The friend says they trust no one else with their child. Several qualified child and adolescent providers practice nearby. What is the **BEST** response?

**A.** Agree, and keep the friendship separate from sessions
**B.** Agree once the teen and friend sign informed consent
**C.** Decline and give names of other local providers
**D.** Assess the teen once, then refer to another provider

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Keeping the friendship out of sessions is hard to do in practice. The worker would hold private information about the teen while socializing with the parent, and either relationship could strain the other.

**B** **Incorrect** (trap: Misreads law or ethics). Informed consent is required for any treatment, but it does not remove a conflict. Signed forms leave the overlapping roles, and the risks to confidentiality and objectivity, exactly where they were.

**C** **Correct.** Social workers avoid dual relationships when there is a risk of exploitation or harm and the relationship can reasonably be avoided (NASW Code 1.06(c)). Treating a close friend's child would mix friendship with a clinical role, threaten the teen's confidentiality and make honest feedback to the parent difficult. Because qualified providers are nearby, declining and offering referrals serves the teen best. It beats D because even one assessment session starts the dual relationship and makes the teen tell their story twice (Reamer, 2020).

**D** **Incorrect** (trap: Good but not best). A single assessment can feel like a helpful bridge, but it already places the worker in a clinical role with a friend's child. It also means the teen repeats their story when a direct referral is available.

**Takeaway:** When a dual relationship can reasonably be avoided, decline and refer, even when a friend or relative asks.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships: avoidable overlapping roles
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 11. INT-1176 (rev 3) · Best answer: A

In an emergency department, a social worker meets with a 52-year-old patient who fell on icy steps after a party. A screening questionnaire shows 5 or 6 drinks on most weekend nights, and brief follow-up questions point to at-risk drinking rather than an alcohol use disorder. What should the social worker do **NEXT**?

**A.** Discuss how alcohol relates to the injury and advise cutting down
**B.** Give the patient a handout on low-risk drinking limits
**C.** Complete a full biopsychosocial assessment before discussing alcohol

**Why each option is right or wrong**

**A** **Correct.** SBIRT pairs screening with a brief intervention matched to risk. For at-risk drinking without signs of an alcohol use disorder, the clinician gives personal feedback, advises cutting down and asks for the patient's view, often in one short talk (NIAAA, 2005). Screening and brief follow-up questions place this patient in the at-risk range, and the injury opens a teachable moment. It beats C because SBIRT acts on the screening result now; a long assessment delays the conversation and may never happen in a busy emergency department.

**B** **Incorrect** (trap: Good but not best). Written information on low-risk limits is a useful add-on, and busy departments often lean on it. On its own, though, it skips the short conversation, with personal feedback, clear advice and the patient's own view, that makes a brief intervention work.

**C** **Incorrect** (trap: Right action, wrong time). The brief assessment SBIRT calls for is already done. A full biopsychosocial assessment fits when a disorder or referral is likely; here it delays the conversation while the injury makes the link to drinking easy to see.

**Takeaway:** After screening shows risky drinking without signs of a disorder, deliver a brief intervention now; save specialty referral for likely disorders.

**Sources:** NIAAA (2005). Helping Patients Who Drink Too Much: A Clinician's Guide, Steps 2-3: assess for disorder, then advise and assist (at-risk drinking)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 12. ASM-1688 (rev 1) · Best answer: B

An agency is planning a caregiver stress group for adult children caring for parents with dementia. Most of the caregivers who have enrolled speak Cantonese and have limited English. The agency's standard curriculum is written and taught in English. Which planning choice is **MOST** likely to make the group effective?

**A.** Run the group in English with a trained interpreter present
**B.** Have a Cantonese-speaking facilitator lead the group in Cantonese
**C.** Translate handouts for Cantonese-speaking members and teach in English

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A trained interpreter is the right fallback when no bilingual facilitator is available. In a support group, though, it keeps members talking through a third party and gives up the stronger effect of native-language delivery.

**B** **Correct.** Culturally adapted interventions outperform unadapted ones, and in a meta-analysis, interventions delivered in clients' native language (when it was not English) were about twice as effective as those delivered in English (Griner & Smith, 2006). A facilitator leading the group in Cantonese lets caregivers speak with each other directly, which is central to group support. It beats A because a trained interpreter provides access and is the right fallback when no bilingual facilitator exists, but members would then talk through a third party (NASW Standards for Cultural Competence, Standard 9).

**C** **Incorrect** (trap: Misuses the technique). Translated handouts help, but teaching in English still leaves caregivers with limited English unable to follow or join the discussion. It adapts the materials without adapting the group itself.

**Takeaway:** When planning services for clients with limited English, delivering the intervention in their native language improves effectiveness beyond interpretation or translated materials.

**Sources:** Griner, D., & Smith, T. B. (2006). Culturally adapted mental health interventions: A meta-analytic review. Psychotherapy: Theory, Research, Practice, Training, 43(4), 531-548, abstract: native-language effect; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

**Area** II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.4 Culture in intervention planning · **Qualifier** MOST LIKELY · **Cognitive** application · **Difficulty** 2 of 3

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## 13. INT-1692 (rev 2) · Best answer: B

A crisis stabilization program manager reviews the monthly budget report at midyear. Overtime spending is 40% above its year-to-date budget, and the program is projected to end the year in deficit. The executive director asks for a plan by next week. What should the manager do **FIRST**?

**A.** Draft a staffing schedule that caps overtime spending

**B.** Find out what is driving the overtime spending

**C.** Ask the board to raise the overtime budget

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). A schedule that limits overtime may well be part of the plan. Drafted before the cause is known, it may cap hours a crisis program needs for safe coverage, or miss a driver such as vacancies or rising admissions that calls for a different fix.

**B** **Correct.** Fiscal oversight compares actual spending with the budget and then analyzes the reasons for any variance before choosing a response (Kettner, Moroney & Martin, 2022). Overtime could reflect vacancies, rising admissions, sick leave or scheduling practices, and each calls for a different fix. Knowing the cause lets the manager propose a plan that controls cost without endangering crisis coverage. It beats C because a budget increase may be justified, but the board will rightly expect an explanation of the variance first.

**C** **Incorrect** (trap: Right action, wrong time). More funding may be warranted, for example if admissions have risen. Requested before the variance is analyzed, it lacks the justification decision makers need and may pay for a problem that scheduling could fix.

**Takeaway:** When spending departs from budget, analyze the cause of the variance before cutting, shifting funds or requesting more.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12: Budgeting for control, management and planning

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 14. ASM-0111 (rev 1) · Best answer: B

At a follow-up visit, a client says, 'I stopped drinking 2 months ago. I go to support meetings twice a week and keep no alcohol at home. Weekends are still hard.' Which stage of change **MOST** likely describes this client?

**A.** Preparation
**B.** Action
**C.** Maintenance

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Preparation means intending to change soon and taking small first steps, such as setting a quit date. This client has already stopped drinking and changed their routines, which places them past preparation.

**B** **Correct.** In the action stage, the person is actively changing the behavior and their surroundings, usually during roughly the first 6 months of change (Prochaska & DiClemente, 1983). This client stopped drinking 2 months ago, attends meetings and keeps alcohol out of the home. It beats C because maintenance refers to sustaining change beyond about 6 months, and this change is still recent and effortful. Stage-matched work means reinforcing progress and problem-solving hard moments such as weekends (SAMHSA TIP 35, Ch. 7).

**C** **Incorrect** (trap: Wrong model or concept). Maintenance is sustaining a change over time, generally after about 6 months, with a focus on preventing recurrence. At 2 months, with weekends still a struggle, the client is in the active early phase instead.

**Takeaway:** Action is recent, effortful change (under about 6 months); maintenance is sustaining change beyond that; preparation is planning to act soon.

**Sources:** SAMHSA TIP 35 (2019), Ch. 7: From Action to Maintenance: Stabilizing Change; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, whole article
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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### 15. INT-1535 (rev 2) · Best answer: D

In a SOAP progress note, which section holds the social worker's clinical interpretation of what the client reported and what the worker observed?

**A.** Subjective
**B.** Objective
**C.** Plan
**D.** Assessment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). The Subjective section records the client's own statements and reported experience, often in quotes. The worker's interpretation of those statements belongs in Assessment, so readers can separate data from judgment.

**B** **Incorrect** (trap: Wrong model or concept). The Objective section holds observable facts such as appearance, behavior, test scores and collateral information. Interpretation goes beyond what was observed, so it belongs in Assessment.

**C** **Incorrect** (trap: Wrong model or concept). The Plan section lists next steps such as referrals, tasks and the next appointment. It follows from the assessment but does not contain the interpretation itself.

**D** **Correct.** SOAP notes, which grew out of Weed's problem-oriented record, separate four kinds of content: Subjective (the client's report), Objective (observed data), Assessment (the professional's interpretation of those data) and Plan (next steps) (Weed, 1968). The worker's clinical interpretation therefore goes in Assessment. It beats A because the interpretation draws on what the client reported, but the Subjective section holds the client's statements themselves, not the worker's judgment about them. Keeping data and judgment apart shows which conclusions rest on which facts (Kagle & Kopels, 2008).

**Takeaway:** SOAP: Subjective is the client's report, Objective is observed data, Assessment is the worker's interpretation, Plan is next steps.

**Sources:** Weed, L. L. (1968). Medical records that guide and teach. New England Journal of Medicine, 278(11), 593-600, problem-oriented progress notes; Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, content and language of records

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 16. ASM-1604 (rev 2) · Best answer: B

A client with bipolar I disorder has spent 12 days on an inpatient unit for mania. The client now sleeps through the night and voices no thoughts of harming anyone, but still has racing thoughts at times while a new medication is adjusted. The client lives with a supportive partner and wants to go home. Which aftercare plan is **BEST**?

**A.** Continue inpatient care until every symptom resolves

**B.** Step down to a partial hospitalization program

**C.** Discharge home with weekly outpatient therapy

**D.** Transfer to a residential treatment program

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Inpatient care is for imminent danger, grave disability or a need for round-the-clock monitoring. None of these remains, and waiting for every symptom to clear keeps the client in a more restrictive setting than needed.

**B** **Correct.** Level-of-care decisions aim for the most integrated, least restrictive setting that can still meet the client's needs (Olmstead v. L.C., 1999). This client no longer needs 24-hour care: sleep has returned, there are no thoughts of harm and a supportive partner is at home. Residual racing thoughts during a medication change, though, call for close monitoring several days a week. Partial hospitalization provides structured treatment most of the day while the client sleeps at home. It beats C because weekly therapy is too thin to catch early relapse signs while the medication is still being adjusted.

**C** **Incorrect** (trap: Ignores risk). Weekly therapy is the right target once mood is stable on the new medication. Stepping down that far now leaves days between contacts while racing thoughts persist, so early signs of relapse could be missed.

**D** **Incorrect** (trap: Good but not best). Residential treatment adds 24-hour living in a facility, which this client does not need with a supportive partner at home. It is more restrictive than the clinical picture calls for and would delay the return home.

**Takeaway:** Choose the least restrictive level of care that still meets current clinical needs; step down gradually while symptoms remain during medication changes.

**Sources:** Olmstead v. L.C., 527 U.S. 581 (1999)
**Area** II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 17. ETH-1026 (rev 1) · Best answer: B

**PRETEST** — unscored on this form; do not count it

A 16-year-old comes to a community health clinic without a parent and asks the social worker for help with daily drinking. The teen does not want a parent contacted and is ready to start today. What should the social worker do **FIRST**?

**A.** Call a parent to get consent before any counseling
**B.** Determine whether state law lets the teen consent alone
**C.** Begin counseling, since the teen is asking for help

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). In states that let minors consent to substance use treatment, calling a parent without the teen's agreement could breach confidentiality and drive the teen away. Parental consent is the right route only where state law requires it.

**B** **Correct.** Minors usually need a parent's consent, but many states let minors consent on their own to substance use or mental health care, and the rules vary. The first step is to learn which rule applies, because it decides who may consent and who controls information about the care (45 CFR 164.502(g)(3); NASW Code 1.03(a)). B beats C because starting today may be lawful, but only if the state allows minor consent. It beats A because calling a parent may override protections the teen has under that law.

**C** **Incorrect** (trap: Right action, wrong time). Starting today is the right move if state law allows the teen to consent alone. Beginning before checking risks treating a minor without valid consent in a state that requires a parent's permission.

**Takeaway:** When a minor seeks care alone, determine whether state law permits minor consent before deciding whom to involve.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.502(g)(3); NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 18. ETH-0003 (rev 1) · Best answer: A

A social worker hears many clients describe abuse and violence. Over time, the social worker develops intrusive images, avoidance and hyperarousal. Which term describes this pattern?

**A.** Secondary traumatic stress
**B.** Occupational burnout
**C.** Countertransference reaction

**Why each option is right or wrong**

**A** **Correct.** Secondary traumatic stress is the set of PTSD-like symptoms, such as intrusive images, avoidance and hyperarousal, that come from indirect exposure to other people's traumatic experiences (Bride, 2007). The symptoms in the question match it directly. It differs from B, burnout, which builds from chronic workload and organizational stress and shows up as exhaustion, cynicism and reduced effectiveness rather than trauma symptoms. Recognizing it matters ethically: the 2021 Code's Purpose section treats self-care as essential to competent practice.

**B** **Incorrect** (trap: Wrong model or concept). Burnout grows out of chronic job demands and organizational stress. Its hallmarks are emotional exhaustion, depersonalization and a reduced sense of accomplishment, as described by Maslach, rather than intrusive trauma images.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference is the social worker's emotional reaction to a particular client, often shaped by the worker's own history. It can occur alongside secondary traumatic stress, but it does not name this trauma-symptom pattern.

**Takeaway:** Trauma symptoms from hearing clients' trauma are secondary traumatic stress; exhaustion and cynicism from workload are burnout.

**Sources:** Bride, B. E. (2007). Prevalence of secondary traumatic stress among social workers. Social Work, 52(1), 63-70, whole article; Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, burnout dimensions; NASW Code of Ethics (2021), Purpose (self-care paragraph)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 19. ASM-1121 (rev 1) · Best answer: B

A school social worker notices that a 9-year-old has stopped going outside at recess and now wears long sleeves on hot days. When asked, the student says, 'I don't want to get darker. Lighter skin is prettier.' What should the social worker do **FIRST**?

**A.** Reassure the student that their skin color is beautiful
**B.** Ask where the student heard ideas about skin color
**C.** Plan a class lesson on diverse beauty standards

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Warm affirmation has a place, but offering it first skips learning what the child has been told and by whom. Reassurance alone may also signal that the topic is closed.

**B** **Correct.** Assessment comes before intervention, and self-image forms through messages from family, peers, media and community (Hepworth et al.). Colorism, the favoring of lighter skin, is spread partly through media images and the beauty industry (Hunter, 2007). The student's avoidance of the sun and new long sleeves show the belief already shapes behavior, so the first step is learning where the message comes from: relatives, classmates, shows or social media. It beats A because reassurance before understanding the source can feel dismissive and leaves the message unaddressed.

**C** **Incorrect** (trap: Right action, wrong time). A lesson on diverse beauty may help later, especially if the message comes from peers at school. Planning it first assumes the source before the student has named it.

**Takeaway:** When a child voices negative beliefs about their appearance, first assess where the messages come from before reassuring or intervening.

**Sources:** Hunter, M. L. (2007). The persistent problem of colorism: Skin tone, status, and inequality. Sociology Compass, 1(1), 237-254, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring the problem and its sources

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.10 Influences on self-image and body image · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 20. ETH-1047 (rev 1) · Best answer: C

An adult client seen in outpatient care after a painful divorce describes recurring thoughts of hurting the former spouse that 'come out of nowhere' at night and frighten the client. The client has given their handgun to a sibling for safekeeping and is eager to keep working on the anger. What is the **BEST** response?

**A.** Warn the former spouse about the client's thoughts

**B.** Arrange a hospital evaluation of dangerousness today

**C.** Intensify treatment with a written safety plan

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A warning is a protective action reserved for a serious threat of violence against an identifiable person. Disclosing unwanted thoughts the client fears and is working on breaches confidentiality and may end the treatment that is lowering the risk.

**B** **Incorrect** (trap: Skips assessment). Hospital evaluation fits when risk is imminent, as with intent, a plan or a refusal to stay safe. Here the client has moved the gun away and seeks help, so hospitalization would be more restrictive than the risk calls for.

**C** **Correct.** The duty to protect arises from a serious threat of violence against an identifiable person (Tarasoff, 1976). This client's thoughts are unwanted and frightening, and the client has already moved the gun away, so the risk is best managed within treatment: more frequent sessions and a written plan that keeps the gun with the sibling. C beats A because warning the former spouse breaches confidentiality without a serious threat (NASW Code 1.07(c)) and could undo the treatment that is reducing risk.

**Takeaway:** Unwanted violent thoughts without intent call for intensified treatment and means safety; warnings are for serious threats against identifiable people.

**Sources:** Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); NASW Code of Ethics (2021), 1.07(c)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 21. ASM-1014 (rev 2) · Best answer: B

In a telehealth intake, a 34-year-old client asks for an ADHD diagnosis to get medication. The client describes poor focus and missed details at work that began 18 months ago, soon after starting rotating night shifts. The client earned good grades in school. Which action is **BEST**?

**A.** Administer an adult ADHD self-report rating scale
**B.** Assess childhood symptoms and sleep disruption
**C.** Tell the client the timeline rules out ADHD

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A rating scale can add useful data, but it screens current symptoms and cannot confirm onset before age 12 or rule out sleep loss. It supplements the history rather than replacing it.

**B** **Correct.** DSM-5-TR requires several ADHD symptoms before age 12, present in two or more settings, and not better explained by another condition. Problems that began with rotating night shifts raise sleep disruption as a possible cause, and the client's childhood history has not been explored; good grades alone do not settle it. Assessing both tests the request directly against the criteria. It beats A because a self-report scale measures current symptoms but cannot establish childhood onset or separate ADHD from sleep loss.

**C** **Incorrect** (trap: Skips assessment). Many adults are diagnosed with ADHD whose childhood symptoms went unrecognized, sometimes despite good grades. Ruling it out before taking a childhood history is premature.

**Takeaway:** ADHD requires several symptoms before age 12; with adult-onset problems, assess childhood history and other causes such as sleep loss before diagnosing.

**Sources:** DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 22. INT-1135 (rev 2) · Best answer: B

A 74-year-old caring at home for a spouse with Parkinson's disease has worked with the social worker for 6 weeks on relaxation, sleep habits, and short daily breaks. The spouse now needs help turning and toileting several times a night, and the caregiver sleeps about 3 hours. The caregiver says, 'I can't keep this up, but letting strangers in feels like breaking my promise to care for my spouse myself.' What should the social worker do **FIRST**?

**A.** Teach a new set of relaxation exercises for the caregiver
**B.** Explore the caregiver's feelings about accepting outside help
**C.** Suggest the caregiver ask a physician about a sleep aid
**D.** Help arrange overnight in-home respite so the caregiver can sleep

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Relaxation skills were taught and practiced for 6 weeks. The barrier now is hands-on care through the night that leaves little time to sleep, which more coping techniques cannot fix.

**B** **Correct.** Exploring ambivalence comes before arranging help the client is unsure about. The caregiver both sees the need ('I can't keep this up') and feels that outside help breaks a promise, so a respite referral made now may be refused or cancelled. Talking through what help would mean, including the spouse's wishes and what kind of help might feel acceptable, lets the caregiver work through the conflict and choose the support (Miller & Rollnick, 2023). It beats D because overnight respite is the likely next step, but arranging it before hearing the caregiver's reservations sidesteps an unresolved choice.

**C** **Incorrect** (trap: Good but not best). A physician visit for the caregiver's own health is reasonable, but a sleep aid does not solve being woken repeatedly for care tasks and could make it harder to respond safely at night.

**D** **Incorrect** (trap: Right action, wrong time). Overnight respite fits the gap: 6 weeks of coping strategies cannot replace sleep lost to nightly care, and respite is a core caregiver support service (Older Americans Act, National Family Caregiver Support Program). It is the likely next step, but the caregiver has just voiced a conflict about outside help, and arranging it before exploring that risks a service the caregiver will not use.

**Takeaway:** When an overloaded caregiver is torn about outside help, explore the reservations first; then arrange respite the caregiver can accept.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Ambivalence: exploring both sides; Older Americans Act, National Family Caregiver Support Program, 42 U.S.C. 3030s-1(b)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 23. ETH-0011 (rev 1) · Best answer: A

In the third session, a Black client tells the social worker that an earlier comment, 'You're so articulate,' felt like a racial stereotype. The social worker listens without interrupting until the client finishes. What should the social worker do **NEXT**?

**A.** Acknowledge the impact of the comment and apologize
**B.** Explain that the comment was meant as a compliment
**C.** Offer to transfer the client to a different social worker

#### Why each option is right or wrong

**A** **Correct.** When a client names a microaggression, repair starts with taking responsibility for the impact rather than defending intent. Acknowledging the harm and apologizing models accountability and protects the alliance (Sue et al., 2007; NASW Code 1.05(c)). The social worker can then invite more feedback and reflect on their own bias outside the session. It beats B because explaining good intentions shifts attention to the social worker's feelings and implies the client misread the comment, which compounds the harm.

**B** **Incorrect** (trap: Good but not best). Explaining intent centers the social worker and tells the client their perception is wrong. Praise that implies surprise at a Black person's verbal skill is a recognized microaggression, whatever the intent (Sue et al., 2007).

**C** **Incorrect** (trap: Hands off too soon). Offering a transfer avoids the repair work and can signal that the relationship cannot survive honest feedback. A transfer can be discussed later if the client wants one, but it is a poor first response to feedback.

**Takeaway:** When a client names a microaggression, acknowledge the impact and take responsibility first; don't defend intent.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 24. INT-1586 (rev 1) · Best answer: B

A county commissioner asks a jail diversion program to show whether the money it saves the county exceeds what it costs. The program director assigns the task to a social worker whose evaluation training covers outcome tracking but not converting outcomes into dollar values. Which action is **BEST**?

**A.** Report cost-effectiveness as cost per person diverted
**B.** Consult an evaluator experienced in cost-benefit analysis
**C.** Decline the task because cost studies are outside social work

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Cost per person diverted is a cost-effectiveness measure. It is useful for comparing programs with the same outcome, but it does not put the outcome in dollars, so it cannot show whether savings exceed costs.

**B** **Correct.** Asking whether savings exceed costs calls for cost-benefit analysis, which expresses both costs and outcomes, such as jail days avoided, in dollars (Rossi, Lipsey & Henry, 2019). Assigning dollar values takes specific skills, and social workers should take on tasks based on existing competence or the intent to gain it (NASW Code 4.01(a)). Consultation lets the work be done well. This beats A because cost per person diverted is a different analysis that leaves the commissioner's question unanswered.

**C** **Incorrect** (trap: Hands off too soon). Evaluating programs is part of social work practice (NASW Code 5.02(a)). Declining leaves the county's question unanswered when consultation would let the social worker complete the task competently.

**Takeaway:** Questions about whether savings exceed costs require cost-benefit analysis; when it exceeds your skills, get expert consultation instead of substituting another measure.

**Sources:** Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 10: Assessing the Economic Efficiency of Programs; NASW Code of Ethics (2021), 4.01(a); NASW Code of Ethics (2021), 5.02(a)
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 25. INT-1558 (rev 1) · Best answer: D

In a parenting group, a parent says that when their 5-year-old screams in the checkout line, the parent buys the child candy and the screaming stops at once. Over several months, the parent has started buying candy as soon as the child begins to fuss. Which learning principle **MOST** likely explains the change in the parent's behavior?

**A.** Positive reinforcement
**B.** Extinction
**C.** Positive punishment
**D.** Negative reinforcement

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). This describes what happens to the child, who receives candy after screaming, so screaming is strengthened. The question asks about the parent, whose behavior grows because the screaming ends, not because the parent gains something.

**B** **Incorrect** (trap: Wrong model or concept). Extinction weakens a behavior by withholding its reinforcer. Here the parent's candy buying is increasing, and the child's screaming is still rewarded, so nothing is being extinguished.

**C** **Incorrect** (trap: Wrong model or concept). Positive punishment adds something unpleasant to reduce a behavior. The parent's candy buying is becoming more frequent, not less, and nothing aversive follows it.

**D** **Correct.** In negative reinforcement, a behavior becomes more frequent because it ends something unpleasant. Buying candy ends the child's screaming, so the parent's giving in grows stronger, which is why the parent now buys candy at the first fuss (Patterson, 1982). It beats A because positive reinforcement describes the child's side of the cycle: the child gains candy, so screaming increases. Seeing this coercive cycle helps parents understand why giving in feels effective in the moment yet makes tantrums more likely.

**Takeaway:** When giving in stops a child's tantrum, the parent's giving in is negatively reinforced; this coercive cycle strengthens both behaviors.

**Sources:** Patterson, G. R. (1982). Coercive Family Process. Castalia, coercion cycle in parent-child interaction
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 26. ETH-1642 (rev 1) · Best answer: A

A school social worker meets with a 16-year-old who arrived in the country alone and now lives with an adult sponsor. The teen falls asleep in class. After gentle questions, the teen explains working nights at a factory 6 days a week to repay a large travel debt to the sponsor, who keeps the teen's identity papers. The teen adds, 'In my country, young people help the family.' What should the social worker do **NEXT**?

**A.** Contact the child abuse hotline about suspected trafficking
**B.** Meet with the sponsor to discuss cutting the work hours
**C.** Ask the teen more about family expectations back home
**D.** Treat the work as a cultural norm and adjust school supports

**Why each option is right or wrong**

**A** **Correct.** A minor working long night hours to repay a debt to a sponsor who holds the teen's identity papers shows recognized signs of labor trafficking; debt bondage is named in the federal definition (22 U.S.C. 7102(7), (11)(B)). Federal law treats a child identified as a trafficking victim as a victim of child abuse (42 U.S.C. 5106g(b)(1)), and reasonable suspicion calls for a report to the child abuse hotline run by child protective services. It beats C because the teen has already described the work and its cultural meaning; family values about helping do not explain withheld papers and a debt owed to the sponsor.

**B** **Incorrect** (trap: Ignores risk). Talking with caregivers is often part of addressing a teen's overwork. Here the sponsor holds the debt and the papers, so alerting the sponsor could lead to threats, moving the teen or hiding evidence before child protective services can act.

**C** **Incorrect** (trap: Right action, wrong time). Exploring cultural meaning is respectful when facts are unclear. The teen has already described the debt, the hours and who holds the papers, so more questions repeat a finished step, delay protection and can feel like disbelief.

**D** **Incorrect** (trap: Skips assessment). Many families expect teens to contribute income, and that alone is not abuse. A debt owed to the sponsor and withheld identity papers go beyond a cultural norm and point to exploitation of a youth whose migration status adds vulnerability.

**Takeaway:** A working minor who owes a debt to a caregiver who holds the minor's papers shows trafficking signs; report to child protection without alerting the caregiver.

**Sources:** Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(7), (11)(B); Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106g(b)(1)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 27. ASM-1051 (rev 2) · Best answer: C

An adult in an intensive outpatient program for moderate alcohol use disorder has cut drinking from daily to 2 evenings a week over 4 weeks. The client also has PTSD from an assault 2 years ago, with nightmares and avoidance that the client links to drinking. The client has stable housing, no withdrawal signs and no suicidal thoughts, and asks to start trauma treatment now. What is the social worker's **BEST** recommendation?

**A.** Defer trauma therapy until 6 months of sobriety
**B.** Refer to a separate trauma clinic after the program ends
**C.** Begin trauma-focused therapy alongside current care

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Requiring a long stretch of sobriety before trauma treatment was once common practice, but trauma-focused therapy is safe and effective alongside substance use treatment. Delay fits only when acute instability, such as withdrawal or crisis, prevents engagement.

**B** **Incorrect** (trap: Hands off too soon). Sending PTSD care elsewhere after the program ends splits and delays treatment. Separate, sequential care is weaker than integrated

care and leaves untreated the nightmares and avoidance that the client ties to drinking.

**C** **Correct.** Current guidance holds that a co-occurring substance use disorder does not rule out trauma-focused psychotherapy for PTSD, and integrated care that treats both conditions together is preferred. This client is engaged, has cut drinking, has stable housing, shows no withdrawal or suicide risk, and links drinking to trauma symptoms, so adding trauma-focused therapy now fits. It beats A because waiting months for sobriety leaves untreated the symptoms that drive drinking (VA/DoD, 2023; SAMHSA, 2021).

**Takeaway:** Co-occurring substance use does not rule out trauma-focused PTSD therapy; treat both together when the client is stable enough to engage.

**Sources:** VA/DoD (2023). Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder. U.S. Department of Veterans Affairs & Department of Defense, Treatment recommendations: trauma-focused psychotherapy with co-occurring conditions; SAMHSA (2021). Advisory: Substance Use Disorder Treatment for People With Co-Occurring Disorders (PEP20-06-04-006), Key Messages; Practice Principles of Integrated Treatment

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 28. ASM-0001 (rev 1) · Best answer: B

**PRETEST** — unscored on this form; do not count it

Which bruise location is **MOST** concerning for physical abuse in an older adult?

**A.** Front of the lower leg
**B.** Upper arm
**C.** Back of the hand
**D.** Forearm below the elbow

**Why each option is right or wrong**

**A** **Incorrect** (trap: Near-miss diagnosis). Shins strike furniture, stairs and walkers often. Lower-leg bruises are among the most common accidental bruises in older adults and are weak evidence of abuse on their own.

**B** **Correct.** Research on older adults found that bruises on the upper arm, head, neck, torso, buttocks and soles of the feet are more often linked to abuse. Bruises on the shins, the forearm below the elbow and the back of the hand are common after accidental bumps and falls (Mosqueda, 2015). Upper-arm bruises can come from being grabbed or shaken. It beats D because the forearm below the elbow is a typical site for bruises from catching oneself in a fall or bumping into objects. A worrying location calls for careful, private assessment, not a conclusion.

**C** **Incorrect** (trap: Near-miss diagnosis). The backs of the hands bruise easily with fragile aging skin and everyday bumps. This location is more often accidental than abusive (Mosqueda, 2015).

**D** **Incorrect** (trap: Near-miss diagnosis). The forearm below the elbow is a typical site for accidental bruising, for example when someone catches themself during a fall. Location alone points away from abuse here, though the full history still matters.

**Takeaway:** In older adults, bruises on the upper arm, head, neck or torso raise abuse concern; shins, forearms and hands are usually accidental.

**Sources:** Mosqueda, L. (2015). Forensic Markers of Elder Abuse. NAPSA/NCPEA Research to Practice Brief, bruise location findings
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 3 of 3

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## 29. ETH-1169 (rev 3) · Best answer: B

A parole officer asks a reentry program social worker how a client is 'doing overall.' The client's release covers only attendance at mandated treatment, and the client also attends an optional peer group. After reviewing the release and the Code and talking with a supervisor, the social worker tells the officer that the client attends all mandated sessions and explains this choice to the client. What should the social worker do **NEXT**?

**A.** Ask a second colleague whether they agree
**B.** Record the options weighed, consultation and reasoning
**C.** Set a date to review how the decision works out

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A second opinion adds little once a supervisor has been consulted and the decision has been made and carried out. More consultation fits when a dilemma remains unresolved, not after a reasoned decision is in place.

**B** **Correct.** Ethical decision-making models end with documenting the decision process and then monitoring its effects (Reamer, 2018). The social worker has identified the issue, reviewed the release and the Code, consulted a supervisor, decided and acted. The record should now show the options considered, the consultation and the reasons for sharing only mandated attendance, as the framework advises, and the Code asks that records be sufficient to support future services (NASW Code 3.04(b)). It beats C because reviewing the outcome comes after documentation; without a record there is nothing to evaluate against, and the reasoning is lost if questions arise later.

**C** **Incorrect** (trap: Right action, wrong time). Reviewing how the decision works out is the final step of the process, and it should happen. It comes after the decision and its reasoning are documented, so there is a record to evaluate against.

**Takeaway:** After resolving an ethical dilemma, document the options, consultation and reasoning, then monitor the outcome.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: decide, document, monitor; NASW Code of Ethics (2021), 3.04(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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### 30. INT-1030 (rev 1) · Best answer: C

A teacher refers a 10-year-old to the school social worker for anger management. The student has thrown books and torn up worksheets several times this month. The teacher's log shows that most outbursts happen when the class reads aloud, and the student's reading grades have dropped all year. What should the social worker do **FIRST**?

**A.** Enroll the student in a small anger management skills group
**B.** Teach the student a calm-down routine to use during reading
**C.** Request an evaluation of the student's reading skills
**D.** Set up a daily reward chart for calm behavior

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A skills group assumes the problem is poor anger regulation. The pattern points to a reading difficulty that sets off the outbursts, so starting a group skips the assessment that would change the plan. A group may help once the cause is addressed.

**B** **Incorrect** (trap: Right action, wrong time). A calm-down routine is a reasonable support and could run alongside other steps. As the first step, though, it manages the outburst while the likely cause, struggling to read in front of classmates, goes unexamined.

**C** **Correct.** Anger work starts by identifying the events that trigger the anger (Reilly & Shopshire, 2019). The log ties the outbursts to reading aloud, and falling reading grades suggest an unmet learning need; the outbursts may help the student escape a task they cannot do in front of peers. Requesting a reading evaluation, which the school can initiate, targets the likely cause (IDEA, 34 CFR 300.301). It beats B because a calm-down routine is useful and may be added, but alone it treats the outburst while the reading problem keeps producing it.

**D** **Incorrect** (trap: Good but not best). Rewards for calm days can shape behavior, but they don't address why reading time sets off the outbursts. A student who cannot do the task may keep failing to earn the reward, which adds frustration.

**Takeaway:** Before teaching anger control, look at what triggers the anger; an unmet need, such as a learning problem, may be driving it.

**Sources:** Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), Session 2: events and cues; Individuals with Disabilities Education Act regulations, 34 CFR 300.301(b)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.20 Helping clients manage anger · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

### 31. ASM-0002 (rev 1) · Best answer: C

A social worker in a rehabilitation facility meets with an 82-year-old patient recovering from hip surgery. Staff report that the patient, who was alert and oriented last week, has become confused over the past 2 days. The patient's attention drifts, and the patient says insects are crawling on the wall. What should the social worker do **FIRST**?

**A.** Administer a cognitive screening to document the decline
**B.** Meet with family to discuss memory care placement
**C.** Notify the medical team of the sudden change in mental status
**D.** Request a psychiatric consultation for new-onset psychosis

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Screening can document the deficits, but it delays the urgent step. Acute confusion with fluctuating attention needs medical evaluation first, and formal cognitive testing during delirium gives misleading results.

**B** **Incorrect** (trap: Skips assessment). Planning memory care assumes a lasting neurocognitive disorder. The rapid onset points to delirium, which often clears when the cause is treated, so placement talk is premature and may be unnecessary.

**C** **Correct.** Confusion that develops over hours to days, with drifting attention and visual hallucinations, in an older patient after surgery strongly suggests delirium. Delirium is a medical emergency with treatable causes such as infection, medication effects or dehydration (DSM-5-TR; Inouye et al., 2014). The first step is making sure the medical team evaluates the change right away. It beats A because a cognitive screen adds information, but scoring the confusion does not get a possible medical emergency treated.

**D** **Incorrect** (trap: Near-miss diagnosis). Visual hallucinations with acute confusion in an older medical patient point to a medical cause rather than a primary psychotic disorder. Psychiatry may help later, but the medical team must look for underlying causes first.

**Takeaway:** Sudden confusion with fluctuating attention in an older adult suggests delirium, a medical emergency; get medical evaluation first.

**Sources:** DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium; Inouye, S. K., Westendorp, R. G. J., & Saczynski, J. S. (2014). Delirium in elderly people. The Lancet, 383(9920), 911-922, whole article
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 32. INT-1656 (rev 1) · Best answer: A

In home hospice, a patient with end-stage heart failure and the patient's partner have both been told the illness is terminal. In separate visits, each tells the social worker they understand the situation. Together, they talk only about next summer's garden and change the subject whenever either mentions the hospice nurse. Which concept **MOST** likely describes this pattern?

**A.** Mutual pretense awareness
**B.** Open awareness
**C.** Denial

**Why each option is right or wrong**

**A** **Correct.** Glaser and Strauss (1965) described awareness contexts around dying. In mutual pretense, the dying person and others all know death is coming, yet in each other's presence they act as though the topic does not exist, often to protect one another. Here each partner privately confirms understanding, but together they steer to future plans and away from hospice. It beats B because open awareness requires that the shared knowledge be acknowledged openly between them, which is exactly what they avoid.

**B** **Incorrect** (trap: Wrong model or concept). Open awareness means everyone knows and can speak openly about dying. Each partner knows, but they avoid the subject with each other, so the knowledge is shared without being open.

**C** **Incorrect** (trap: Wrong model or concept). Denial is the Kübler-Ross stage in which a person rejects the reality of the illness (Kübler-Ross, 1969). Both partners privately say they understand the situation, so their avoidance protects each other rather than refusing the facts.

**Takeaway:** In mutual pretense, the dying person and family both know but avoid the subject together; private acknowledgment separates it from denial.

**Sources:** Glaser, B. G., & Strauss, A. L. (1965). Awareness of Dying. Aldine, awareness contexts: mutual pretense; Kübler-Ross, E. (1969). On Death and Dying. Macmillan, stages: denial

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 33. ASM-1652 (rev 1) · Best answer: B

A 15-year-old has spent 10 months in a residential treatment program and has met most treatment goals. Discharge home is set for 3 weeks from now, and the youth has said little about it. In the past week, the youth has broken curfew twice and picked fights with peers, behaviors not seen in months. Staff ask the social worker whether discharge should be delayed. What is the **BEST** course of action?

**A.** Postpone discharge until the behavior settles for several weeks

**B.** Help the youth talk about leaving while keeping the planned date

**C.** Keep the planned date and review program rules with the youth

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). Delaying discharge can fit when new behavior shows real danger or lost skills. Here the timing points to a reaction to the coming separation. Postponing may reward the behavior and prolong a restrictive placement the youth no longer needs.

**B** **Correct.** Old problems often resurface as an important ending approaches; Hepworth et al. describe clients reintroducing earlier difficulties as termination nears. A youth who has met most goals, says little about leaving, and acts out only as discharge nears is most likely reacting to losing the program's relationships and structure. Opening that conversation, and using the remaining weeks to prepare for the move, protects the youth's gains. It beats A because delaying discharge treats an expected transition reaction as treatment failure and can teach the youth that acting out postpones hard endings.

**C** **Incorrect** (trap: Good but not best). Holding to the date is reasonable, but answering the behavior with a rules review treats it as simple misconduct. It misses the likely meaning, the youth's unspoken feelings about leaving, so the stress behind the behavior goes unaddressed.

**Takeaway:** Regression as discharge nears often reflects anxiety about the ending; address the transition directly before treating it as treatment failure.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: clients' reactions, including return of earlier problems

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.13 Effects of out-of-home placement (hospital, foster, residential, justice) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 34. ASM-1565 (rev 1) · Best answer: A

**PRETEST** — unscored on this form; do not count it

In a primary care clinic, a patient with 2 years of low back pain is referred to the social worker after normal imaging. The patient lost a warehouse job, sleeps poorly, and avoids walking for fear of reinjury. The patient asks, 'So the doctor thinks the pain is all in my head?' Which response is **MOST** helpful?

**A.** Explain that real pain is affected by mood, stress and activity

**B.** Reassure the patient that the pain has a purely physical cause

**C.** Explain that emotional stress is the likely cause of the pain

**Why each option is right or wrong**

**A** **Correct.** The biopsychosocial model of chronic pain holds that pain is a real experience shaped by interacting biological, psychological and social factors, including mood, sleep, fear of movement and job loss (Gatchel et al., 2007). Normal imaging does not mean the pain is imagined. Explaining this validates the patient and opens the door to work on sleep, avoidance and stress. It beats B because insisting on a purely physical cause would discourage the psychosocial work that can reduce pain and disability.

**B** **Incorrect** (trap: Good but not best). Reassurance may ease the patient's worry, but claiming a purely physical cause contradicts the biopsychosocial view and makes it harder to accept help with sleep, fear of movement and job loss.

**C** **Incorrect** (trap: Wrong model or concept). Saying stress causes the pain repeats the mind-body split the patient fears and can feel dismissive. Psychological factors influence pain alongside biology rather than replacing it as the explanation.

**Takeaway:** Chronic pain is real and shaped by interacting biological, psychological and social factors; explain it that way, not as either physical or mental.

**Sources:** Gatchel, R. J., Peng, Y. B., Peters, M. L., Fuchs, P. N., & Turk, D. C. (2007). The biopsychosocial approach to chronic pain: Scientific advances and future directions. Psychological Bulletin, 133(4), 581-624, biopsychosocial model of chronic pain

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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### 35. INT-1119 (rev 2) · Best answer: B

In an early psychosis program, a 22-year-old client who smokes cannabis daily asks whether it is 'really that bad.' The social worker first asks what the client has heard, then briefly shares research on heavy cannabis use and psychotic relapse. What should the social worker do **NEXT**?

**A.** Advise the client to cut back before the next visit
**B.** Ask what the client makes of this information
**C.** Ask whether the client is willing to cut back now
**D.** Give a handout that summarizes the research findings

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Advice given before hearing the client's reaction tends to provoke push-back and turns a shared exploration into a lecture. Advice can be offered with permission once the client has weighed the information.

**B** **Correct.** Elicit-provide-elicit ends by asking for the person's reaction to the information just shared (Miller & Rollnick, 2023). The worker has already asked what the client knew and offered the research, so the next step is to learn how the client interprets it. The response shows whether the information landed, brings out doubts, and lets the client draw their own conclusions about cannabis, which supports autonomy. It beats D because a handout can reinforce the message later, but handing it over now skips the client's reaction, which shapes everything that follows.

**C** **Incorrect** (trap: Misuses the technique). Asking about commitment has a place in MI, but jumping to it now skips the client's reaction to what was just shared and tilts the talk toward compliance. A yes-or-no question this early invites a quick 'no' or an empty 'yes.'

**D** **Incorrect** (trap: Right action, wrong time). A written summary can support memory and fits near the end of the conversation. Giving it now skips hearing how the client understands the information and what they make of it.

**Takeaway:** Close an information exchange by inviting the client's interpretation before any advice, so the conclusions remain the client's own.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Exchanging information: elicit-provide-elicit
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 36. INT-0113 (rev 1) · Best answer: B

A client plans to ask a supervisor at work for a schedule change to care for a relative. The client says they freeze whenever they speak with people in authority. Which approach is **BEST** for helping the client prepare during the session?

**A.** Write a script for the client to read to the supervisor
**B.** Rehearse the request with the social worker acting as supervisor
**C.** Offer to call the supervisor and explain the client's needs

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A script can help a client organize thoughts and may be a useful first draft. Without rehearsal, though, the client still meets the freezing response the first time they speak with the supervisor.

**B** **Correct.** Role play lets a client practice a hard conversation in a safe setting, get feedback and try again until the skill feels manageable. Successful practice is a mastery experience, the strongest source of self-efficacy (Bandura, 1977). Because this client freezes with authority figures, rehearsing with the social worker in the supervisor's role targets the exact moment that goes wrong. It beats A because a written script supplies words but no practice delivering them under pressure, which is where the client struggles.

**C** **Incorrect** (trap: Boundary slip or collusion). Calling for the client does a task the client can learn to do and may reinforce the belief that they cannot handle authority figures. Advocacy fits when a client is unable to act for themselves.

**Takeaway:** When a client fears a specific conversation, role-play rehearsal builds skill and self-efficacy better than scripts or acting on the client's behalf.

**Sources:** Bandura, A. (1977). Self-efficacy: Toward a unifying theory of behavioral change. Psychological Review, 84(2), 191-215, performance accomplishments as a source of efficacy
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 37. ASM-1575 (rev 1) · Best answer: B

At a case management intake in a homeless shelter, a client sighs, pushes the forms aside and says, 'The shelter staff, the benefits office, now you. Everybody wants paperwork from me.' The social worker wants to build engagement early in the meeting. Which response is **BEST**?

**A.** 'So several offices need forms from you right now.'
**B.** 'It sounds like you're worn out by everyone's demands.'
**C.** 'I'll keep my part of the paperwork as short as I can.'

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). This paraphrase captures the facts but misses the feeling under them. It is accurate and harmless, yet it tells the client the worker heard the list of offices, not the frustration. Paraphrasing content fits better when the client's message is mainly informational.

**B** **Correct.** Reflection of feeling names the emotion behind the client's words and shows empathic understanding, which lowers defensiveness and builds engagement (Hepworth et al.). The sigh, pushing the forms away and 'everybody wants' all signal weariness and frustration, so naming that feeling meets the client where they are. It beats C because a promise about shorter paperwork jumps to problem solving before the client feels understood, and it may be a promise the worker cannot keep.

**C** **Incorrect** (trap: Good but not best). Offering to lighten the load is kind and may help later, but it skips past the client's feelings and moves straight to fixing. It can also sound like a bargain. Practical adjustments fit once the worker has acknowledged how the client feels.

**Takeaway:** When a client's words carry strong emotion, reflect the underlying feeling before paraphrasing content or offering solutions.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication: reflecting feelings
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 38. INT-1154 (rev 2) · Best answer: A

In week 5 of a middle school group for students who have a parent in prison, a member's grandparent calls the school social worker. At lunch, another group member told classmates about the member's parent's charges, which the member had shared in group. The member is upset and now refuses to return. What should the social worker do **FIRST**?

**A.** Meet with the member to hear the impact and plan next steps
**B.** Address the breach at the next session and restate group rules
**C.** Meet first with the member who shared the information

**Why each option is right or wrong**

**A** **Correct.** When a group confidence is broken, the leader first learns how the affected member was harmed and involves that member in deciding how the breach will be handled. This member is hurt and withdrawing, and discussing the breach in group without the member's input could expose the information again. Leaders seek members' agreement to keep confidences but cannot guarantee it (NASW Code 1.07(f)). It beats B because repairing group norms is the right next step, but it should follow the affected member's safety and wishes (Corey, Corey & Corey, 2018).

**B** **Incorrect** (trap: Right action, wrong time). Processing the breach with the group and restating confidentiality is necessary and will likely follow. Doing it before talking with the injured member risks reopening the disclosure without consent and losing a member who has not yet been heard.

**C** **Incorrect** (trap: Right action, wrong time). The member who disclosed will need a conversation about the breach and its consequences, and some leaders start there. Doing it first puts the harmed member second and risks decisions about the breach before that member has been heard or consulted.

**Takeaway:** After a group confidentiality breach, start with the affected member: learn the harm and plan together before processing the breach in group.

**Sources:** NASW Code of Ethics (2021), 1.07(f); Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, confidentiality in groups; handling breaches
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 39. INT-1132 (rev 1) · Best answer: C

**PRETEST** — unscored on this form; do not count it

Which PTSD treatment has the client write an impact statement and then examine 'stuck points,' such as self-blame for the trauma?

**A.** Prolonged exposure
**B.** EMDR
**C.** Cognitive processing therapy
**D.** Trauma-focused CBT

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Prolonged exposure reduces PTSD by having the client repeatedly recount the trauma memory and approach safe but avoided reminders. It works mainly through exposure, not through an impact statement and stuck-point work.

**B** **Incorrect** (trap: Wrong model or concept). EMDR pairs recall of trauma images with bilateral stimulation, such as eye movements, across eight phases. It does not use a written impact statement or structured work on stuck points.

**C** **Correct.** Cognitive processing therapy (CPT) is a structured, usually 12-session cognitive treatment for PTSD. The client writes an impact statement about why the trauma happened and how it changed beliefs about self, others, and the world, and the work then targets stuck points, such as self-blame, that keep recovery from moving forward (Resick, Monson & Chard, 2017). It beats A because prolonged exposure relies on repeated exposure to memories and reminders rather than on examining written beliefs.

**D** **Incorrect** (trap: Wrong model or concept). TF-CBT is designed for children and adolescents with their caregivers, building coping skills before a trauma narrative. It is not the protocol organized around an impact statement and stuck points.

**Takeaway:** Cognitive processing therapy targets trauma-related stuck points, such as self-blame, starting from a written impact statement.

**Sources:** Resick, P. A., Monson, C. M., & Chard, K. M. (2017). Cognitive Processing Therapy for PTSD: A Comprehensive Manual. Guilford Press, impact statement; stuck points
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 40. ETH-0102 (rev 1) · Best answer: B

At intake at a community mental health agency, a client says they do not read well and asks the social worker to 'just show me where to sign' the consent form. The client speaks English fluently. What is the **MOST** effective way for the social worker to obtain informed consent?

**A.** Read the form aloud, then have the client sign it
**B.** Explain each part plainly and check understanding
**C.** Point to the signature line, as the client requested

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Reading the form aloud helps a client who reads little, but it is a one-way step. Without checking what the client understood and inviting questions, the signature may still fall short of informed consent.

**B** **Correct.** Valid consent requires that the client understands the purpose, risks, costs, alternatives and the right to refuse. When literacy is limited, the Code calls for a detailed verbal explanation and steps to ensure comprehension (NASW Code 1.03(a), 1.03(b)). Explaining each element in plain language and asking the client to say it back confirms understanding. It beats A because reading the form aloud gives the client the words but does not show that the client grasped them or had a chance to ask questions.

**C** **Incorrect** (trap: Misreads law or ethics). A signature is not the same as informed consent. Honoring the request to skip the explanation leaves the client agreeing to services they may not understand, which the Code does not treat as valid consent.

**Takeaway:** Informed consent means understanding, not a signature; with limited literacy, explain in plain words and confirm comprehension.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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## 41. ETH-1154 (rev 1) · Best answer: A

At the final session of a planned termination, a client who completed treatment for an eating disorder asks the social worker to shred their whole record that day. The client wants a 'clean slate' and worries that a future employer might see it. The agency must keep records for a set number of years. What is the **BEST** response?

**A.** Explain the retention rule and how the record is protected
**B.** Remove the most sensitive notes from the record
**C.** Agree to shred it once the client signs a request

**Why each option is right or wrong**

**A** **Correct.** Records must be kept after services end for as long as laws, agency policy and contracts require (NASW Code 3.04(d)), and this agency has such a requirement, so a client's wish cannot shorten it. The helpful response explains the rule and addresses the real worry: the record is stored securely, is not released to an employer without the client's authorization except as law requires, and will be destroyed securely when the period ends (1.07(s)). It beats C because a signed request, though it honors the client's wish, does not change a legal or policy retention duty.

**B** **Incorrect** (trap: Misreads law or ethics). Pulling sensitive notes would leave an incomplete record that no longer matches the services provided (3.04(a)) and breaks the retention duty for the removed material, whatever the client's later preference.

**C** **Incorrect** (trap: Misreads law or ethics). A written request shows respect for the client's choice, but consent cannot override a required retention period. Shredding early would violate agency policy and possibly law, and could leave the client without records they may need later.

**Takeaway:** Clients cannot waive record retention rules; explain the required period, how the record is protected and when it will be destroyed.

**Sources:** NASW Code of Ethics (2021), 3.04(d); NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 1.07(s)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 42. ETH-1715 (rev 1) · Best answer: C

A 15-year-old with one Black parent and one white parent tells the school social worker that classmates keep saying the teen 'has to pick a side.' The teen calls themself mixed and feels close to both families' traditions. A teacher has suggested the teen would fit in better by identifying as Black, since that is how others see them. What is the **MOST** appropriate response?

**A.** Encourage the teen to identify as others perceive them
**B.** Explore which side of the family feels most like home
**C.** Affirm the teen's right to define their own identity

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Others' perceptions shape the bias a multiracial teen may face, and talking about that can help. Telling the teen to adopt that label lets outsiders define identity and dismisses the teen's own sense of self.

**B** **Incorrect** (trap: Misuses the technique). Identity exploration is valuable, but this question assumes the teen must choose one family's side, the same either-or framing the classmates impose. The teen has already said they feel close to both.

**C** **Correct.** Multiracial people have the right to name their own identity, which may differ from how others see them and may shift across settings (Root, 1996). The teen already describes an identity rooted in both families, so the worker's role is to affirm that self-definition and help the teen respond to pressure, recognizing clients as experts on their own culture (NASW Code 1.05(c)). It beats A because how others perceive the teen matters for preparing for bias, but it does not decide who the teen is; urging a single label repeats the classmates' pressure.

**Takeaway:** Multiracial clients may define their own identity, even when it differs from how others see them; affirm it rather than urging one label.

**Sources:** Root, M. P. P. (Ed.). (1996). The Multiracial Experience: Racial Borders as the New Frontier. Sage Publications, A bill of rights for racially mixed people; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 43. ETH-1567 (rev 1) · Best answer: C

What is the **MOST** appropriate way for a social worker in solo private practice to prepare for sudden death or incapacity?

**A.** Ask a family member to contact clients if needed
**B.** Tell clients at intake to seek another provider if needed
**C.** Name a colleague in a written plan to notify and refer clients

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Family members are not bound by professional confidentiality and may not know how to handle records or referrals. Having them contact clients risks revealing who is receiving services (NASW Code 1.07(t)).

**B** **Incorrect** (trap: Good but not best). Mentioning the possibility at intake is honest but does little when it happens. Clients would learn of the death or incapacity late, with no help to transfer and no one managing their records.

**C** **Correct.** Social workers make reasonable efforts to keep services going if they become unavailable through illness, disability or death (NASW Code 1.15). They also take precautions to protect confidentiality if they become incapacitated or die (1.07(t)). A written plan that names a qualified colleague, explains how to reach clients and records, and arranges referrals meets both duties; such a plan is often called a professional will. It beats B because a notice at intake leaves clients to find care alone in a crisis, with no one to inform them or manage their records.

**Takeaway:** Private practitioners plan ahead for death or incapacity by naming a qualified colleague to notify clients, secure records and arrange referrals.

**Sources:** NASW Code of Ethics (2021), 1.15; NASW Code of Ethics (2021), 1.07(t)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 44. INT-1514 (rev 1) · Best answer: C

At a family resource center, a parent lists four concerns: finishing a GED, a car that needs repairs, a child's behavior at school, and an electricity shutoff notice for Friday. It is January, the home is heated by electricity, and an infant lives there. The parent says the child's school behavior worries them most. What should the social worker do **FIRST**?

**A.** Begin with the child's school behavior that the parent named
**B.** Ask the parent to rank the four concerns by importance
**C.** Work with the parent to stop the utility disconnection
**D.** Build one plan that covers the four concerns together

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Starting with the client's top concern is usually right and builds engagement. Here, a heating shutoff in winter with an infant at home is an imminent safety risk with a fixed deadline, so it comes first.

**B** **Incorrect** (trap: Right action, wrong time). Asking the client to rank concerns is a sound step when priorities are unclear. The parent has already named a top concern, and one problem carries a safety deadline that ranking would not change.

**C** **Correct.** When a client has many problems, the worker helps prioritize, usually starting with what matters most to the client, but threats to safety and basic needs come first. A January power shutoff in an electrically heated home with an infant is an imminent health risk due in days, and it is concrete and solvable (Maslow, 1943; Hepworth et al.). It beats A because starting where the client is remains the general rule; the worker can explain the urgency, deal with the shutoff, then turn to the school concern.

**D** **Incorrect** (trap: Misuses the technique). Tackling everything at once overwhelms the client and loses focus. Problems are partialized and taken one at a time, starting with the most urgent.

**Takeaway:** Help clients prioritize by their own concerns, except that imminent threats to safety or basic needs are addressed first.

**Sources:** Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, hierarchy of needs; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: prioritizing problems
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.4 What the social worker does in problem solving · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 45. INT-1140 (rev 2) · Best answer: B

A client with schizophrenia in a supported employment program has had five job interviews without an offer. The client says the interviews 'went fine' and cannot say what might have gone wrong. What should the social worker do **FIRST** to help the client improve?

**A.** Teach a standard set of tips for answering interview questions
**B.** Ask the client to role-play an interview while the worker observes
**C.** Help the client request feedback from the interviewers

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Teaching interview skills will likely be part of the plan, but choosing tips before seeing how the client interviews guesses at the problem. Assessment through observation comes first.

**B** **Correct.** Social skills training starts with assessment, and a role-played situation lets the worker see the actual behaviors, such as eye contact, voice, and length of answers, that self-report misses (Bellack et al., 2004). The client says the interviews went fine and cannot name a problem, so asking more questions is unlikely to reveal the gaps. Observing a practice interview pinpoints which skills to teach. It beats A because generic tips may target skills the client already has and miss the ones costing offers.

**C** **Incorrect** (trap: Good but not best). Employer feedback can be informative, so this is a reasonable thought. Many interviewers decline to give it, though, and waiting for replies delays help, while the worker can see the client's interview behavior directly in a practice interview today.

**Takeaway:** When a client cannot describe what goes wrong in a social situation, use role-play to observe and assess skills before teaching.

**Sources:** Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, Ch. 3: Assessment of social skills
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 46. ASM-1184 (rev 2) · Best answer: C

A client at a community health center paid a smuggler to bring them across the border. On arrival, the smuggler said the client now owed twice the agreed fee, took the client's identity papers, and placed the client in a restaurant kitchen that pays nothing until the debt is cleared. The client says, 'I chose to come, so this is my problem.' Which term **MOST** likely describes the client's current situation?

**A.** Human smuggling
**B.** Wage theft
**C.** Labor trafficking

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Smuggling is the illegal movement of a person across a border, usually with that person's agreement, and the arrangement typically ends on arrival. It describes how the client entered, which is why it is tempting, but not the exploitation happening now.

**B** **Incorrect** (trap: Wrong model or concept). Wage theft is an employer's failure to pay wages owed, a labor-law violation. It misses the coercion: seized papers and an inflated debt that the client must work off.

**C** **Correct.** Trafficking is exploitation of a person through force, fraud or coercion to obtain labor or commercial sex; it does not require movement and can follow a smuggling arrangement (DHS Blue Campaign). Holding a person to work off an inflated debt is debt bondage, and taking identity papers adds coercion (TVPA, 22 U.S.C. 7102). It beats A because smuggling describes the client's agreed border crossing, while the forced unpaid work now is trafficking, whatever the client agreed to earlier.

**Takeaway:** Smuggling is a crime against a border; trafficking is exploitation of a person, and a smuggled person can become a trafficking victim.

**Sources:** U.S. Department of Homeland Security, Blue Campaign (2024). What Is Human Trafficking? (brochure), Human trafficking vs. human smuggling; Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(3), (7), (11)(B)
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.15 Exploitation across the lifespan: financial, immigration-related, trafficking · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 47. ETH-1002 (rev 1) · Best answer: D

A social worker in private practice receives a release, signed by a client, from the client's attorney. The client has sued another driver after a car crash and claims emotional distress. The release covers treatment records from the past year. Those records describe a past affair that the client may not want raised in court. What is the **MOST** appropriate response?

**A.** Assert privilege until a judge orders release of the records
**B.** Withhold the pages about the affair to protect the client
**C.** Send the records the release covers to the attorney today
**D.** Review the records' contents with the client, then release them

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Misreads law or ethics). Asserting privilege is right when no authorization exists. Here the holder of the privilege has waived it in writing for these records, so the social worker cannot assert it against the client's wishes.
- **B** **Incorrect** (trap: Overrides the client's choice). Removing pages the client authorized overrides the client's decision and alters what is produced. If the client wants a narrower release after reviewing the content, the client can change it; the social worker cannot decide that alone.
- **C** **Incorrect** (trap: Good but not best). The release is valid, so sending the records is lawful. It is weaker because the client may not realize the records include the affair, and the Code asks workers to explain disclosures and their consequences first when feasible.
- **D** **Correct.** The client, not the social worker, holds privilege, and a valid signed release waives it for the records it covers (Jaffee v. Redmond, 1996; NASW Code 1.07(b)). The worker honors that choice but, when feasible, tells the client about the disclosure and its possible consequences beforehand (NASW Code 1.07(d)). Reviewing the contents lets the client decide with full knowledge, including whether to narrow the release. D beats C because sending at once is lawful but skips the step that lets the client weigh the sensitive content.

**Takeaway:** When a client waives privilege, honor the waiver, but first help the client understand what the released records contain.

**Sources:** Jaffee v. Redmond, 518 U.S. 1 (1996); NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(d)

**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 48. ETH-1175 (rev 1) · Best answer: C

A new state law requires mental health providers to notify a parent before a minor's fourth counseling session. A social worker in a school-based counseling program believes the law will keep some teens from seeking help. The program's attorney has confirmed that the law applies, and several students are already in counseling. What is the **BEST** action for the social worker to take?

- **A.** Keep the current practice until a court rules on the law
- **B.** Stop taking new student clients until the law changes
- **C.** Comply with the law and advocate for changing it
- **D.** Ask an ethics committee whether to follow the law

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Misreads law or ethics). Ignoring a law that the program's attorney says applies exposes students and the program to harm, including possible loss of the service itself. Disagreeing with a law is a reason to work for its change, not to disregard it while services continue.
- **B** **Incorrect** (trap: Hands off too soon). Closing the door to new students removes help from the very teens the social worker wants to protect. Withdrawing services does not resolve the conflict; it shifts the cost of the law onto students who need care.

**C** **Correct.** When law and ethics conflict, the NASW Code asks social workers to make a responsible effort to resolve the conflict consistently with its values (Purpose) and to advocate for changes in legislation (6.04(a)). The attorney has confirmed the law applies, so the social worker complies, tells current students about the new limit on confidentiality before it affects them (1.07(e)), and works to change the law. It beats D because the Code reserves consultation for conflicts with no reasonable resolution; here the legal question is settled and a resolution exists.

**D** **Incorrect** (trap: Good but not best). The Code calls for consultation when a reasonable resolution does not appear possible. Here the legal question is already answered and a workable resolution exists, so another round of review delays compliance and leaves current students uninformed about the change.

**Takeaway:** When a law conflicts with ethical values, comply, tell affected clients about new limits, and advocate through legitimate channels to change it.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 1.07(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 49. ETH-1675 (rev 1) · Best answer: B

**PRETEST** — unscored on this form; do not count it

A social worker at a community mental health agency learns they need surgery in 3 weeks, followed by a recovery period that could last several months. The worker carries 26 clients, several with recurrent depression, and wants to keep the medical details private. Colleagues at the agency have openings. How should the social worker **BEST** handle the coming absence with clients?

**A.** Wait to tell clients until the surgeon gives a firm medical timeline
**B.** Tell clients soon about a medical leave and review options with each
**C.** Share the diagnosis so clients understand why the worker will be away
**D.** Transfer every client to a colleague now, before the surgery date

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Waiting for an exact return date leaves clients little time to prepare and shortens the window for planning coverage. An uncertain length is itself information clients need. The notice can say the length is unknown and be updated later.

**B** **Correct.** Social workers who expect to interrupt services should notify clients promptly and seek transfer, referral or continuation based on each client's needs and preferences (NASW Code 1.17(e); 1.15). Three weeks allows time to talk with each client about interim coverage, transfer or pausing, and the worker can call it a medical leave without sharing details. It beats D because moving everyone at once ignores client preferences: some clients may want a colleague now, while others may prefer to wait, with coverage, for the worker's return.

**C** **Incorrect** (trap: Boundary slip or collusion). Clients need to know that the worker will be away and what their options are, not the worker's diagnosis. Sharing medical details

shifts attention to the worker and can burden clients, especially those with depression, without improving their choices.

**D** **Incorrect** (trap: Hands off too soon). Arranging colleague coverage is part of the plan, but transferring every client now, without asking, removes the client's voice from the decision. Transfer fits when a client chooses it or when the absence becomes indefinite.

**Takeaway:** When services will be interrupted, notify clients promptly and plan continuation around each client's needs and preferences, without oversharing personal details.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 50. ASM-1684 (rev 1) · Best answer: D

At a first treatment-planning meeting at a community mental health center, a 20-year-old discharged from a psychiatric unit 2 weeks ago says the only goal is to return to college full time next month. The client adds that this 'matters more than anything.' The discharge summary lists steady sleep and medication routines as early priorities. What should the social worker do **FIRST**?

**A.** Explain that full-time enrollment next month is too soon
**B.** Record the college goal in the plan exactly as the client stated it
**C.** Break the goal into steps, such as one course next term
**D.** Ask what attending college full time would mean to the client

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Feasibility matters, but declaring the goal unrealistic at the first meeting is directive and risks the alliance. Concerns about timing are better raised together once the client's reasons are understood.

**B** **Incorrect** (trap: Good but not best). Recording the goal as stated honors the client's wishes, but it skips understanding and shaping the goal together. A plan built this way may set the client up for an early setback.

**C** **Incorrect** (trap: Right action, wrong time). Partializing is likely the right next step and may lead to a single course as a first objective. Proposed before exploring why the goal matters, it can read as the worker overriding the client's aim.

**D** **Correct.** Goals are negotiated with the client, and understanding what a goal means comes before narrowing or partializing it (Hepworth et al.). The client's strong investment signals that college carries weight, perhaps hope or proof of recovery, that the plan can build on, for example by linking sleep and medication routines to the client's own aim. Asking what attending would mean starts there; recovery planning is person-driven (SAMHSA, 2012). It beats C because breaking the goal into steps is a sound next move, but offered first it can feel like a downgrade before the client feels understood.

**Takeaway:** In goal setting, first understand what a client's stated goal means to them, then shape and partialize it together.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals: selecting and defining goals with clients; SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery is person-driven

**Area** II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 51. ASM-1112 (rev 2) · Best answer: A

At a disaster recovery center 7 weeks after a wildfire destroyed their home, a client reports nightmares about the flames most nights, avoids driving past the burned area, and has missed work for 3 weeks. A brief screen at the center confirms these symptoms, and the client reports no suicidal thoughts. What should the social worker do **NEXT**?

**A.** Refer the client for a trauma-focused mental health evaluation

**B.** Reassure the client that these reactions are expected after disasters

**C.** Rescreen the client in a month to see if symptoms fade

**Why each option is right or wrong**

**A** **Correct.** Most early stress reactions after a disaster fade, which is why psychological first aid, not therapy, is the standard first response (Brymer et al., 2006). Symptoms that last more than a month and impair functioning, though, meet the duration DSM-5-TR uses for PTSD and call for full evaluation and evidence-based treatment. At 7 weeks, with frequent nightmares, avoidance and 3 weeks of missed work, this client needs that referral. It beats B because reassurance fits the first days after a disaster, not persistent symptoms that disrupt work.

**B** **Incorrect** (trap: Right action, wrong time). Normalizing reactions helps in the first days after a disaster, when most distress is expected and fades. At 7 weeks, with frequent symptoms and missed work, reassurance alone leaves a likely treatable condition unaddressed.

**C** **Incorrect** (trap: Good but not best). Watchful waiting suits mild reactions soon after an event. These symptoms have already lasted beyond a month and disrupt work, so another month of waiting delays effective care.

**Takeaway:** Early disaster reactions are expected, but symptoms lasting over a month with impaired functioning warrant referral for trauma evaluation.

**Sources:** Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and overview; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 52. INT-1690 (rev 2) · Best answer: C

A community agency banned text messaging with clients to reduce privacy risk. A compliance review a year later finds that many case managers text clients from personal phones anyway. In interviews, staff explain that many clients have prepaid phones with few voice minutes and answer texts but not calls. Which policy response is **BEST**?

**A.** Reinforce the ban and discipline staff texting from personal phones
**B.** Survey clients about how they prefer the agency to reach them
**C.** Allow texting on agency phones with set safeguards

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Enforcement fits when staff break a workable rule for convenience. Here the review shows the rule conflicts with how clients can be reached; discipline would cut contact with these clients or push texting further out of sight.

**B** **Incorrect** (trap: Right action, wrong time). Asking clients is a sound step when their needs are unknown. Staff interviews already show that clients rely on texts because of limited voice minutes, so a survey delays a needed policy change.

**C** **Correct.** Evaluating a risk policy means checking whether it reduces the risk in practice. This ban has pushed texting onto personal phones, where messages are unsecured, outside the record and beyond agency control, a greater privacy risk than the one the ban targeted. Allowing texting on agency phones with safeguards meets clients' real needs, lets the agency protect the messages (NASW Code 1.07(m)) and keeps staff from exposing personal numbers, as NASW safety guidelines advise (NASW, 2013). It beats A because stricter enforcement ignores why staff text; contact with clients who cannot afford calls would drop or go further underground.

**Takeaway:** When a risk policy produces unsafe workarounds, revise it to meet the legitimate need with safeguards rather than enforcing it harder.

**Sources:** NASW Code of Ethics (2021), 1.07(m); NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 5: Use of Mobile Phones
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 53. ETH-1584 (rev 2) · Best answer: D

A social worker at a regional agency is asked to lead a needs assessment for a neighborhood the agency has served poorly. Past agency programs there drew few participants, and local leaders have told the agency, 'You always decide for us.' Agency leadership wants results within 6 months. What is the **BEST** way for the social worker to design the assessment?

**A.** Use a validated national survey to ensure comparable data
**B.** Analyze the agency's service records to find unmet needs
**C.** Hire an outside research firm to run neutral focus groups
**D.** Form a community advisory group to shape questions and methods

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Validated instruments add rigor and comparison, and an advisory group might choose to include one. Selecting it alone keeps the agency defining the neighborhood's needs, repeating the pattern local leaders named.

**B** **Incorrect** (trap: Good but not best). Agency records show the needs of people who already use services. They miss the many people who stayed away, who are the group the assessment most needs to reach.

**C** **Incorrect** (trap: Hands off too soon). An outside firm may seem neutral, but it moves control to another set of outsiders. People in the neighborhood still have no say in what is asked or how results are used.

**D** **Correct.** Anti-oppressive practice shares power with the people affected, and participatory approaches make community members partners in defining questions, methods and the use of results (Israel et al., 1998). Local leaders say the agency decides for them and past programs went unused, so involving the community in the design addresses both the power imbalance and the likely cause of low use. An advisory group shapes data collection rather than replacing it, so the 6-month window holds. It beats A because a national survey adds rigor but leaves decisions about what matters with the agency (NASW Code 6.02).

**Takeaway:** Anti-oppressive needs assessment shares design power with the affected community, especially where people report being decided for.

**Sources:** Israel, B. A., Schulz, A. J., Parker, E. A., & Becker, A. B. (1998). Review of community-based research: Assessing partnership approaches to improve public health. Annual Review of Public Health, 19, 173-202, whole article; NASW Code of Ethics (2021), 6.02

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 54. ETH-1144 (rev 1) · Best answer: B

In a private conversation, a colleague at a family services agency tells a social worker about completing treatment for alcohol use disorder 8 months ago and attending weekly recovery meetings. The colleague's supervisor rates their work highly, and their clients are making progress. A coworker who overheard urges a report to the licensing board. What is the **MOST** appropriate action?

**A.** Report the colleague's history to the state licensing board

**B.** Respect the colleague's privacy and make no report

**C.** Tell the supervisor so the agency can monitor the colleague

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). A board report fits impairment that harms practice and that the colleague has not addressed, or a report the law requires. Recovery with strong performance meets neither condition, and reporting would break the colleague's confidence.

**B** **Correct.** The duty to act on a colleague's impairment applies when an impairment interferes with practice effectiveness (NASW Code 2.08(a)); formal channels follow when the colleague has not addressed it (2.08(b)). This colleague completed treatment, stays engaged in recovery and practices well, and a history of treatment is not

impairment. Social workers also respect confidential information colleagues share (2.02). B beats C because telling the supervisor treats recovery as a risk and breaks the colleague's confidence when clients are doing well.

**C** **Incorrect** (trap: Misreads law or ethics). Monitoring would make sense if there were signs of impairment at work. Alerting the supervisor here shares a private disclosure and treats recovery itself as the problem.

**Takeaway:** Recovery is not impairment; the duty to act on a colleague begins when an impairment interferes with practice.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b); NASW Code of Ethics (2021), 2.02
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 55. ASM-1021 (rev 1) · Best answer: A

A child welfare social worker reviews the case of a 9-year-old in kinship foster care. Since age 7, the child has been angry and irritable most of every day. Several times a week, the child has severe outbursts at home and at school, such as screaming and throwing chairs, far beyond what peers do. What is the **MOST** likely diagnosis?

**A.** Disruptive mood dysregulation disorder
**B.** Oppositional defiant disorder
**C.** Bipolar I disorder with rapid cycling
**D.** Intermittent explosive disorder

#### Why each option is right or wrong

**A** **Correct.** Disruptive mood dysregulation disorder involves severe, recurrent temper outbursts, about three or more a week, with irritable or angry mood most of the day between them, lasting at least 12 months in two or more settings, with onset before age 10 (DSM-5-TR). Two years of daily irritability and frequent outbursts at home and school fit. It beats B because when a child meets criteria for both, DSM-5-TR directs that only disruptive mood dysregulation disorder be diagnosed; the severe outbursts and constant irritability go beyond defiance.

**B** **Incorrect** (trap: Near-miss diagnosis). Oppositional defiant disorder involves angry mood and defiance, but when severe outbursts and constant irritability meet criteria for disruptive mood dysregulation disorder, that diagnosis is made instead.

**C** **Incorrect** (trap: Near-miss diagnosis). Bipolar I, even with a rapid-cycling pattern, requires distinct manic episodes. Chronic, nonepisodic irritability in a child is the pattern disruptive mood dysregulation disorder was created to describe, partly to curb overdiagnosis of pediatric bipolar disorder.

**D** **Incorrect** (trap: Near-miss diagnosis). Intermittent explosive disorder involves impulsive outbursts with normal mood between them. This child stays irritable most of every day, which points to disruptive mood dysregulation disorder.

**Takeaway:** Frequent severe outbursts plus irritable mood most days between them, for 12+ months starting before age 10, suggests disruptive mood dysregulation disorder.

**Sources:** DSM-5-TR (APA, 2022), Depressive Disorders: Disruptive Mood Dysregulation Disorder
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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## 56. INT-1183 (rev 1) · Best answer: B

Which term describes the vocal qualities that accompany spoken words, such as tone, pitch, pace and volume?

**A.** Kinesics
**B.** Paralanguage
**C.** Proxemics
**D.** Haptics

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Kinesics is the study of body movement, including gestures, posture and facial expression. It is the channel lost on a phone call; vocal qualities remain available there.

**B** **Correct.** Paralanguage, or vocal cues, covers how something is said rather than the words themselves: tone, pitch, rate, volume, pauses and sighs (Knapp, Hall & Horgan, 2014). It matters on the phone and in person, because a flat or rushed voice can contradict a client's words. It beats A because kinesics covers body movement, gesture, posture and facial expression, which are seen rather than heard.

**C** **Incorrect** (trap: Wrong model or concept). Proxemics concerns the use of space and interpersonal distance, such as how close people sit or stand. It says nothing about how the voice sounds.

**D** **Incorrect** (trap: Wrong model or concept). Haptics refers to touch as communication, such as a handshake or a hand on the shoulder. It is a separate nonverbal channel from the voice.

**Takeaway:** Nonverbal channels: paralanguage is the voice (tone, pace, volume), kinesics the body, proxemics space, haptics touch.

**Sources:** Knapp, M. L., Hall, J. A., & Horgan, T. G. (2014). Nonverbal Communication in Human Interaction (8th ed.). Wadsworth, Cengage Learning, Vocal cues; body movement; space; touch
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 57. ASM-1519 (rev 1) · Best answer: A

A 47-year-old client with Down syndrome has lived in a supported apartment for 10 years. Over the past 6 months, staff report that the client gets lost on familiar bus routes, skips steps when cooking and has lost interest in a weekly bowling league. Staff call the client 'stubborn' and want a behavior plan. What should the social worker do **FIRST**?

**A.** Seek a medical and cognitive assessment of the decline
**B.** Create a reward plan for completing daily tasks
**C.** Ask the client how they feel about the recent changes
**D.** Reteach cooking steps using picture-based cues and prompts

**Why each option is right or wrong**

**A** **Correct.** New loss of daily skills in an adult with Down syndrome calls for medical and cognitive evaluation, because these adults face a greatly increased risk of Alzheimer-type dementia, and treatable causes such as thyroid problems, depression or vision loss must be ruled out (Alzheimer's Association). Calling the change stubbornness is diagnostic overshadowing: attributing new problems to the existing disability (Reiss et al., 1982). It beats D because retraining assumes a learning problem and could delay finding a treatable or progressive cause.

**B** **Incorrect** (trap: Skips assessment). A reward plan assumes the client is choosing not to perform tasks. Building it before the decline is evaluated risks treating a medical or neurocognitive change as a behavior problem and delaying care.

**C** **Incorrect** (trap: Good but not best). The client's view matters and should be included, but asking about feelings cannot explain a 6-month loss of skills. Making it the first step delays the evaluation that new decline requires.

**D** **Incorrect** (trap: Right action, wrong time). Visual supports and skill retraining may help later. Starting them first treats the decline as a learning issue before medical causes and dementia have been assessed.

**Takeaway:** New functional decline in an adult with an intellectual disability needs medical and cognitive evaluation; avoid attributing it to the disability or to behavior.

**Sources:** Alzheimer's Association. Differential Diagnosis of Down Syndrome (alz.org professional resources), risk, early signs and conditions to rule out; Reiss, S., Levitan, G. W., & Szyszko, J. (1982). Emotional disturbance and mental retardation: Diagnostic overshadowing. American Journal of Mental Deficiency, 86(6), 567-574, whole article

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.17 Effects of physical and mental ability across the lifespan · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 58. ETH-1043 (rev 2) · Best answer: C

A hospice social worker noticed a colleague nodding off in team meetings and slurring words on a call. Two weeks ago, the social worker raised this privately, and the colleague denied any problem. This week the colleague missed two patient visits and seemed sedated at a family meeting. What should the social worker do **NEXT**?

**A.** Meet with the colleague a second time to urge help
**B.** Keep a log of incidents for a few more weeks
**C.** Take the concerns to the colleague's supervisor

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A second conversation can feel kinder, and talking privately was the right first step. The colleague has already denied a problem and the signs are getting worse, so the Code's next step is the agency channel.

**B** **Incorrect** (trap: Ignores risk). Recording specific incidents is useful, but weeks more of watching while patients miss visits and the colleague appears sedated with families leaves the harm unaddressed.

**C** **Correct.** With direct knowledge of a colleague's impairment, the social worker first talks with the colleague when feasible (NASW Code 2.08(a)), which has already happened. When the colleague does not take adequate steps, the next move is action through appropriate channels such as the employer (NASW Code 2.08(b)). Missed patient visits and apparent sedation show the problem continues. C beats A because repeating a talk the colleague has already rebuffed delays protection for patients and families.

**Takeaway:** If an impaired colleague does not act after a direct conversation, take the concern to the employer or agency channel.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 59. ASM-1017 (rev 2) · Best answer: B

A client is brought to an emergency department with hallucinations and disorganized speech. The client cannot give a history, no records or collateral contacts are available, and the duration of symptoms is unknown. Which diagnosis does DSM-5-TR provide for this situation?

**A.** Other specified schizophrenia spectrum and other psychotic disorder
**B.** Unspecified schizophrenia spectrum and other psychotic disorder
**C.** Schizophreniform disorder, provisional

**Why each option is right or wrong**

**A** **Incorrect** (trap: Near-miss diagnosis). Other specified is used when the clinician knows why criteria for a named disorder are not met and records that reason, such as persistent hallucinations with no other features. An emergency team with almost no history cannot state any such reason.

**B** **Correct.** DSM-5-TR uses an unspecified diagnosis when symptoms of a disorder class cause distress or impairment but there is not enough information for a more specific diagnosis, and it names emergency settings as an example. It beats A because other specified requires the clinician to state the specific reason the presentation does not meet a named disorder, which demands information the team lacks. The diagnosis is revised as history, duration and possible substance or medical causes become known (DSM-5-TR).

**C** **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder requires symptoms for at least 1 month, and the provisional label applies while recovery within 6 months is still uncertain. With no history, even the 1-month minimum is unknown.

**Takeaway:** Use unspecified when information is too limited for a specific diagnosis; use other specified when you can state why criteria are not met.

**Sources:** DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Unspecified Schizophrenia Spectrum and Other Psychotic Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Other Specified Schizophrenia Spectrum and Other Psychotic Disorder
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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## 60. ASM-1511 (rev 2) · Best answer: D

Two days after a home invasion, a client meets with a social worker in a prosecutor's victim services office. The client gives a precise, minute-by-minute account of being tied up and threatened with a gun. Throughout, the client speaks in a flat, even tone, shows no visible emotion and asks what happens next in the case. Which defense mechanism is the client **MOST** likely using?

**A.** Dissociation
**B.** Intellectualization
**C.** Denial
**D.** Isolation of affect

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Dissociation disrupts consciousness, memory or identity, such as gaps in recall or feeling unreal. It is the closest neighbor here, but this client's detailed, continuous memory of the event points to emotion split off from an intact memory.

**B** **Incorrect** (trap: Wrong model or concept). Intellectualization keeps feelings at a distance through general, logical or abstract framing (Di Giuseppe & Perry, 2021). This client recounts concrete personal events in vivid detail, and asking about next steps in the case is a practical question, not abstract analysis. What stands out is the missing feeling, not theorizing.

**C** **Incorrect** (trap: Wrong model or concept). Denial refuses to accept that an event happened or matters. The client fully acknowledges the home invasion and asks about the case, so the reality is accepted while the feelings are kept away.

**D** **Correct.** Isolation of affect separates an idea or memory from the feeling attached to it, so a distressing event is recalled clearly but without emotion (Vaillant, 1992). This client gives a precise, detailed account of a terrifying assault in a flat tone, with memory intact and feeling absent. It beats A because dissociation disrupts memory, awareness or sense of self, and this client's minute-by-minute recall shows continuous memory. Isolation of affect can protect a client early after trauma, so the social worker paces emotional exploration.

**Takeaway:** Isolation of affect recalls distressing events clearly but without feeling; dissociation disrupts memory or awareness; intellectualization retreats into abstract analysis.

**Sources:** Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, isolation of affect; dissociation; Di Giuseppe, M., & Perry, J. C. (2021). The hierarchy of defense mechanisms: Assessing defensive functioning with the Defense Mechanisms Rating Scales Q-Sort. Frontiers in Psychology, 12, 718440, obsessional level: isolation of affects vs. intellectualization
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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## 61. ASM-1012 (rev 2) · Best answer: C

A social worker in private practice completes an intake with a 38-year-old client who reports 2 months of low mood, fatigue and poor concentration. The client also mentions a 15-pound weight gain, feeling cold all the time and thinning hair. The client has not had a physical exam in 5 years. What should the social worker do **NEXT**?

**A.** Begin cognitive behavioral therapy for the depression
**B.** Administer a standardized depression scale to rate severity
**C.** Refer the client to a primary care provider for a medical evaluation

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Cognitive behavioral therapy may well help later. Starting treatment for depression before checking for a medical cause risks missing a treatable illness that could explain the mood symptoms.

**B** **Incorrect** (trap: Good but not best). A depression scale measures how severe the symptoms are, but it cannot show whether a physical illness is causing them. The physical signs call for a medical exam first.

**C** **Correct.** Before a depressive disorder is diagnosed, the social worker considers whether another medical condition could explain the symptoms (DSM-5-TR). Weight gain, constant cold intolerance and thinning hair alongside low mood and fatigue are signs that call for medical evaluation, for example of thyroid function, and the client has had no exam in 5 years. The social worker does not diagnose the medical condition but routes the client to a physician (Kaplan & Sadock). It beats B because a severity score adds detail but cannot tell a medical cause from a primary depression.

**Takeaway:** Depressive symptoms with physical signs and no recent medical exam call for medical evaluation before a depressive diagnosis or treatment.

**Sources:** DSM-5-TR (APA, 2022), Depressive Disorders: Depressive Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Depressive disorders chapter: differential diagnosis (medical conditions)
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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## Section 2

### 62. ETH-1190 (rev 2) · Best answer: A

**PRETEST** — unscored on this form; do not count it

A licensed social worker opens a solo practice and no longer has a supervisor. Which habit is **MOST** appropriate for keeping clinical judgment sound over time?

**A.** Meeting regularly with a consultation group to review cases
**B.** Completing the continuing education hours required for renewal
**C.** Reviewing past supervision notes when a case feels difficult

**Why each option is right or wrong**

**A** **Correct.** Social workers should seek colleagues' advice whenever it serves clients (2.05(a)) and strive to remain proficient (4.01(b)). A solo practitioner without a supervisor loses routine outside feedback on actual cases, so regular case consultation is the habit that catches blind spots and drift in judgment as the work happens. It beats B because continuing education, which 4.01(b) names as one way to stay current, builds general knowledge but gives no feedback on the practitioner's own clinical decisions.

**B** **Incorrect** (trap: Good but not best). Continuing education is required for renewal and is expected by the Code (4.01(b)), so it belongs in any professional's routine. It updates general knowledge but does not examine the practitioner's own cases, so it cannot catch blind spots the way regular consultation can.

**C** **Incorrect** (trap: Good but not best). Looking back at old supervision notes can recall useful advice, but it offers no fresh outside view of current cases. Judgment drifts most when no one else reviews the work, which regular consultation with colleagues addresses directly.

**Takeaway:** Licensed practitioners without supervision should build in regular case consultation; continuing education updates knowledge but gives no feedback on actual cases.

**Sources:** NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 2.05(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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### 63. ASM-1109 (rev 1) · Best answer: B

A teacher asks the school social worker to begin an ADHD referral for an 8-year-old. Over the past month, the student has stopped finishing work, jumps at loud sounds, and acts out hiding under a desk during play. The parent mentions a shooting on their street 6 weeks ago. What should the social worker do **FIRST**?

**A.** Send the ADHD rating scales the teacher requested to the parent
**B.** Assess the student's exposure to violence and reactions
**C.** Refer the student for trauma-focused therapy

**Why each option is right or wrong**

**A** **Incorrect** (trap: Near-miss diagnosis). ADHD symptoms begin before age 12, appear in more than one setting and last at least 6 months. Problems that started this month, right after a shooting, call for trauma assessment before an ADHD workup.

**B** **Correct.** Assessment comes before referral, and attention problems that begin after a frightening event may be trauma reactions. DSM-5-TR lists trouble concentrating and an exaggerated startle among PTSD symptoms and notes that children may replay trauma themes in play. The timing after the shooting, the startle and the hiding play point to trauma exposure, so the social worker first assesses what the student experienced and how they are reacting. It beats C because trauma-focused therapy may be the right outcome, but assessment must establish the need and guide the plan.

**C** **Incorrect** (trap: Right action, wrong time). Trauma-focused therapy is likely appropriate if assessment confirms trauma-related symptoms. Referring first skips the step that shows what the child needs and how severe the reactions are.

**Takeaway:** When new attention or behavior problems follow a frightening event, assess trauma exposure and reactions before pursuing other explanations.

**Sources:** DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder; DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 64. ASM-1582 (rev 1) · Best answer: C

On an inpatient psychiatric unit, the treatment team drafts a discharge plan before meeting with a patient admitted after a manic episode. At the planning meeting, the patient, now calm and organized, says, 'You all already decided everything.' What should the social worker do **FIRST**?

**A.** Walk the patient through each part of the team's draft plan
**B.** Explain why the team chose each recommendation in the plan
**C.** Invite the patient to name the problems that matter most to them

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Reviewing the draft is a needed step, but starting with it confirms the patient's complaint that the plan was made without them. The draft can be reviewed after the patient's own priorities are on the table.

**B** **Incorrect** (trap: Too directive). Explaining the team's reasoning is useful, but leading with it turns the meeting into persuasion. It treats the patient as someone to convince rather than a partner in defining the problems.

**C** **Correct.** Clients take part in identifying problems and goals, and plans built on their priorities are more likely to be followed (Hepworth et al.). The patient is now organized enough to participate and has just named feeling shut out, so the first move is to ask what they want the plan to address, honoring self-determination (NASW Code 1.02). Team concerns are then merged in. It beats A because reviewing the draft first repeats the very exclusion the patient described.

**Takeaway:** Begin intervention planning by asking the client to name the problems and goals that matter to them, then integrate team concerns.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Developing goals collaboratively with clients; NASW Code of Ethics (2021), 1.02
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.3 Engaging clients in problem identification and intervention planning · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 65. ETH-1509 (rev 1) · Best answer: B

A social worker at a community agency is planning a kitchen remodel. The social worker's partner collected bids, and the lowest bid comes from a contractor who is a current client of the social worker. The client does not yet know the social worker is the homeowner. What is the **MOST** appropriate action?

**A.** Hire the client and pay the full bid amount
**B.** Accept one of the other bids instead
**C.** Ask the client in session how the job would feel

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Paying the full price avoids taking a discount, but it still makes the client the worker's paid contractor. Any disagreement about the work could spill into treatment, and the client may feel unable to complain or say no.

**B** **Correct.** Social workers avoid dual relationships with clients when there is a risk of exploitation or harm (NASW Code 1.06(c)). Hiring a current client creates a business tie in which disputes over cost, quality or schedule could damage treatment, and the client may feel unable to object. Unlike an unavoidable small-town overlap, this one is easy to avoid by choosing another bidder. It beats C because raising the job in session would put the client on the spot and turn treatment time toward the worker's own interests (NASW Code 1.06(b)).

**C** **Incorrect** (trap: Good but not best). Asking the client's view sounds respectful, but the condition is already clear: this is an avoidable dual relationship. Raising it would pressure the client and use session time for the worker's own business needs.

**Takeaway:** When a dual relationship with a client is easy to avoid, avoid it rather than try to manage it.

**Sources:** NASW Code of Ethics (2021), 1.06(c); NASW Code of Ethics (2021), 1.06(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 66. INT-1644 (rev 1) · Best answer: C

A patient with schizophrenia, admitted after a suicide attempt, is ready for discharge. The team finds the patient no longer at imminent risk and able to make treatment decisions. The patient declines any outpatient appointment, saying the hospital stay was enough, and plans to return home. Which follow-up plan is **MOST** appropriate?

**A.** Delay discharge until the patient agrees to an appointment
**B.** Give a list of local clinics and close the case at discharge
**C.** Offer to send brief, caring check-in messages after discharge

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). A patient who is no longer at imminent risk and can make treatment decisions may decline aftercare. Holding discharge to extract agreement is coercive and can damage trust the patient may need later.

**B** **Incorrect** (trap: Good but not best). A resource list respects the refusal and is part of any discharge, but closing the case ends all contact during the highest-risk weeks. A low-demand way to stay connected serves the patient better.

**C** **Correct.** The weeks after psychiatric discharge carry high suicide risk, and a capable patient may still decline treatment. Motto and Bostrom (2001) found that brief, nondemanding letters sent periodically to people who refused ongoing care after hospitalization for depression or suicidal behavior were associated with fewer suicides in the following two years. Offering caring contacts respects the patient's choice while keeping a connection through the riskiest period. It beats B because a clinic list and closure meet the minimum but leave no contact at all after a suicide attempt, when follow-up matters most.

**Takeaway:** When a capable patient declines aftercare after a suicidal crisis, offer brief caring contacts that keep a connection without making demands.

**Sources:** Motto, J. A., & Bostrom, A. G. (2001). A randomized controlled trial of postcrisis suicide prevention. Psychiatric Services, 52(6), 828-833, whole article
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 67. ETH-1007 (rev 1) · Best answer: A

**PRETEST** — unscored on this form; do not count it

At the first meeting of an intensive outpatient group for adults recovering from alcohol use disorder, a member asks the social worker, 'Can you promise that nobody here will repeat what I say?' What is the **BEST** response?

**A.** Explain the group's privacy agreement and the limits on enforcing it
**B.** Assure the member that group rules keep everything private
**C.** Suggest the member keep sensitive details out of group

**Why each option is right or wrong**

**A** **Correct.** In group work, the social worker seeks agreement among members to keep shared information confidential and tells them that the worker cannot guarantee every member will honor it (NASW Code 1.07(f)). The worker also explains the worker's own legal limits on confidentiality (NASW Code 1.07(e)). An honest answer lets each member decide what to share. A beats C because advising silence about sensitive topics undercuts the purpose of the group, while an honest explanation protects members and still invites participation.

**B** **Incorrect** (trap: Misreads law or ethics). This promise cannot be kept, because group members are not bound the way professionals are. A false guarantee misleads the member and damages trust if someone later repeats what was said.

**C** **Incorrect** (trap: Good but not best). Members may choose what to share, but steering them away from sensitive material weakens the group. Explaining the agreement and its limits lets each member make an informed choice instead.

**Takeaway:** In groups, ask members to agree to confidentiality, and say honestly that the social worker cannot guarantee others will comply.

**Sources:** NASW Code of Ethics (2021), 1.07(f); NASW Code of Ethics (2021), 1.07(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 68. INT-0005 (rev 1) · Best answer: C

An inpatient social worker is planning discharge for an adult with schizophrenia. The patient has had three admissions this year, each after missing the first outpatient appointment. The patient is stable and agrees to outpatient care. Which discharge step is **BEST**?

**A.** Give the patient a list of clinics to call after discharge
**B.** Extend the stay until the patient shows full insight
**C.** Schedule a clinic visit within 7 days and arrange a warm handoff

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A resource list is better than nothing, but it leaves the patient to make contact alone, the very step that failed three times. This patient needs active linkage.

**B** **Incorrect** (trap: Overrides the client's choice). Keeping a stable, agreeable patient in the hospital is more restrictive than needed, and full insight is not a discharge requirement. The least restrictive setting plus strong linkage fits better.

**C** **Correct.** This patient's history shows that the gap between hospital and clinic is where care breaks down. Scheduling the first visit before discharge, within 7 days, and arranging a warm handoff closes that gap. Seven-day follow-up is a standard quality measure (NCQA HEDIS FUH), and the weeks right after discharge carry sharply raised suicide risk (Chung et al., 2017). It beats A because a list leaves the hardest step, making first contact, to the patient, which is exactly where past discharges failed.

**Takeaway:** For discharge, link actively: book the first follow-up within 7 days and hand off warmly, especially when past transitions failed.

**Sources:** National Committee for Quality Assurance. HEDIS measure: Follow-Up After Hospitalization for Mental Illness (FUH), 7-day follow-up rate; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 69. INT-1089 (rev 1) · Best answer: B

An employee assistance client is struggling after a close friend died of an overdose 3 weeks ago. The friend's family did not invite the client to the funeral, the employer denied bereavement leave because the friend was not a relative, and coworkers have said, 'It was bound to happen.' Which concept **MOST** likely describes the client's experience?

**A.** Anticipatory grief
**B.** Disenfranchised grief
**C.** Prolonged grief disorder
**D.** Ambiguous loss

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Anticipatory grief begins before an expected death, as with a terminal illness. This friend has already died, and the problem is how others respond to the loss, not grieving in advance.

**B** **Correct.** Disenfranchised grief is grief that is not openly acknowledged, socially supported or publicly mourned, as often happens after the death of a friend, an ex-partner or someone who died by overdose (Doka, 1989). Exclusion from the funeral, denied leave because the friend was not family, and blaming remarks all signal that others do not recognize this loss. It beats C because the grief is intense, but at 3 weeks it is far too early for prolonged grief disorder, which requires at least 12 months since the death in adults (DSM-5-TR).

**C** **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder requires persistent, impairing grief at least 12 months after a death in adults. Three weeks after the loss, intense grief is expected; the striking feature here is the lack of social recognition.

**D** **Incorrect** (trap: Wrong model or concept). Ambiguous loss involves uncertainty, such as a missing person, or someone physically present but psychologically absent with dementia. This death is certain; what is missing is social permission to grieve.

**Takeaway:** Disenfranchised grief is grief others do not recognize or support, common after deaths of friends, ex-partners or by overdose.

**Sources:** Doka, K. J. (Ed.). (1989). Disenfranchised Grief: Recognizing Hidden Sorrow. Lexington Books, definition of disenfranchised grief; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 70. INT-1610 (rev 1) · Best answer: A

An associate social worker in a jail mental health program came from a job where a supervisor publicly criticized every mistake. The new supervisor gives mild, specific feedback, yet the associate apologizes repeatedly, leaves errors out of case reviews and braces for reprimand at each meeting. What is **MOST** likely occurring?

**A.** Supervisee transference
**B.** Parallel process
**C.** Supervisor countertransference

**Why each option is right or wrong**

**A** **Correct.** Transference in supervision occurs when a supervisee reacts to the supervisor as to an important earlier figure, often a past authority (Bernard & Goodyear, 2019). The associate meets mild feedback with apologies, hidden errors and expected reprimand, a pattern that fits the harsh former supervisor rather than the current one. It beats B because parallel process involves a client dynamic replayed in supervision, and this pattern traces to a past supervisor and appears across all case reviews.

**B** **Incorrect** (trap: Wrong model or concept). Parallel process is the replay of a client-worker dynamic in the supervisory relationship, or the reverse (St. Arnaud, 2017). Here the pattern follows a past supervisor and appears in every review, not in connection with a particular case.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference would be the supervisor's own reaction to the associate. The stem describes the supervisee's responses, and the supervisor's feedback is mild and specific.

**Takeaway:** A supervisee who reacts to a current supervisor as to a past authority figure shows transference within supervision.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; St. Arnaud, K. O. (2017). Encountering the wounded healer: Parallel process and supervision. Canadian Journal of Counselling and Psychotherapy, 51(2), 131-144, whole article

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 71. INT-0008 (rev 1) · Best answer: A

Which type of evaluation examines whether a program is being delivered as designed while the program is still under way?

**A.** Formative evaluation
**B.** Summative evaluation
**C.** Needs assessment
**D.** Cost-benefit analysis

**Why each option is right or wrong**

**A** **Correct.** Formative, or process, evaluation takes place during implementation. It checks fidelity, meaning whether services reach the intended people in the intended way, so the program can be improved while it runs (Royse, Thyer & Padgett, 2016). Both cues in the question, delivery as designed and timing during operation, point to it. It beats B because summative evaluation comes at the end and judges whether the program achieved its outcomes, not how it is being carried out.

**B** **Incorrect** (trap: Wrong model or concept). Summative, or outcome, evaluation judges results after a program cycle, such as whether goals were met. It does not monitor delivery while the program runs.

**C** **Incorrect** (trap: Wrong model or concept). A needs assessment comes before a program is designed, to identify gaps and the population's needs. It does not examine how an existing program is being delivered.

**D** **Incorrect** (trap: Wrong model or concept). Cost-benefit analysis compares a program's costs and benefits in monetary terms. It answers a question about economic value rather than whether delivery matches the design.

**Takeaway:** Formative (process) evaluation monitors delivery during a program; summative evaluation judges results at the end; needs assessment comes first.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, formative and process evaluation
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 72. ASM-1157 (rev 1) · Best answer: A

In the lobby of a community mental health center, the social worker finds a client who has opioid use disorder slumped in a chair. The client does not respond to a loud voice or a firm knuckle rub on the breastbone. The client's lips look bluish, and they are making slow snoring sounds. Naloxone is kept at the front desk. What should the social worker do **FIRST**?

**A.** Call 911 and give the naloxone
**B.** Place the client on their side and hold naloxone in reserve
**C.** Ask the front desk whether the client took anything

**Why each option is right or wrong**

**A** **Correct.** Unresponsiveness, bluish lips and slow, snoring breaths are signs of opioid overdose; the snoring signals blocked or failing breathing, not sleep. SAMHSA's toolkit directs responders to call 911 and give naloxone, then support breathing and stay with the person (SAMHSA, 2018). It beats C because learning what the client took can help responders, but every minute without naloxone deepens oxygen loss, and the signs already present are enough to act on.

**B** **Incorrect** (trap: Ignores risk). Placing the person on their side protects the airway and is part of overdose care. Holding naloxone back while the client is unresponsive and barely breathing delays the drug that reverses the overdose; it should be given right away.

**C** **Incorrect** (trap: Right action, wrong time). Information about what was taken is useful to emergency staff and can be gathered once help is on the way. Seeking it first delays naloxone when the overdose signs are already clear.

**Takeaway:** Unresponsiveness with slow or snoring breathing and bluish lips means possible opioid overdose: call 911 and give naloxone without delay.

**Sources:** SAMHSA (2018) Opioid Overdose Prevention Toolkit, Five Essential Steps for First Responders
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 73. INT-1617 (rev 1) · Best answer: B

A community agency plans to replace paper charts with an electronic record. Months before any training, the director shares data on missed follow-ups caused by lost paper files, holds staff forums about problems with the current system and invites concerns. In Lewin's model, which stage is the director **MOST** likely working on?

**A.** Refreezing
**B.** Unfreezing
**C.** Moving

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Refreezing stabilizes a change after it is made, through new policies, reinforcement and routine use. The electronic record has not been introduced yet.

**B** **Correct.** Lewin (1947) described planned change in three steps: unfreezing, moving and refreezing. Unfreezing builds readiness by showing that the current way of working is not acceptable and by surfacing concerns before new behavior is introduced. Sharing data on missed follow-ups and inviting staff concerns before any training is unfreezing. It beats C because moving is the stage in which people learn and adopt the new system, such as training and go-live, which has not started.

**C** **Incorrect** (trap: Wrong model or concept). Moving is the transition itself: training staff and beginning to use the new record. The director's work comes before any training begins.

**Takeaway:** In Lewin's model, unfreezing builds readiness by showing the current way is not working; moving is the change; refreezing stabilizes it.

**Sources:** Lewin, K. (1947). Frontiers in group dynamics: Concept, method and reality in social science; social equilibria and social change. Human Relations, 1(1), 5-41, whole article
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 74. ETH-1520 (rev 1) · Best answer: C

During a home visit, an assertive community treatment social worker finds that a client with schizophrenia has refused all food and water for 3 days, believing it is poisoned. The client is dizzy, confused and unsteady when standing, and refuses to go to the hospital. What should the social worker do **FIRST**?

**A.** Explore the client's reasons for refusing the hospital
**B.** Respect the refusal and return tomorrow with sealed food
**C.** Seek emergency evaluation despite the client's refusal

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Exploring the reasons for a refusal is the right first step when risk is uncertain and the client can weigh choices. Here the client is confused after 3 days without fluids, so the condition for overriding the refusal is already met.

**B** **Incorrect** (trap: Misreads law or ethics). Respecting a refusal is correct when a competent client faces no imminent danger. Dehydration with confusion is a serious and imminent risk, and waiting until tomorrow could lead to collapse or death.

**C** **Correct.** Self-determination may be limited when, in professional judgment, a client's actions pose a serious, foreseeable and imminent risk to self (NASW Code 1.02). Three days without fluids, with confusion and unsteadiness, signals a medical emergency driven by psychosis, and the confusion undercuts the client's capacity to decide (NASW Code 1.14). The social worker follows applicable law to obtain emergency evaluation now. It beats A because exploring the refusal fits when risk is unclear; here the risk is established and delay could be fatal.

**Takeaway:** Self-determination yields when a client's choice poses serious, foreseeable and imminent risk to self; then secure emergency evaluation.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.14
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 75. ETH-1608 (rev 1) · Best answer: C

A college student meets with a campus social worker about stress over choosing a major. The student wants to study art, but the parents favor nursing. The student says, 'In my family, big decisions are made together. Going against my parents would bring shame on all of us, and I don't want that.' Which approach is **BEST**?

**A.** Help the student separate personal wishes from parents' wishes
**B.** Invite the family to a session to negotiate the choice
**C.** Help the student weigh options in light of family values

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Separating one's own wishes from others' is central to some Western models of maturity. Here it treats the student's stated value as a problem to fix and pushes an individualist goal the student did not choose.

**B** **Incorrect** (trap: Right action, wrong time). A joint session could help if the student wants it. Arranging it now skips the student's own thinking about how to approach the parents and may feel like pressure from the social worker.

**C** **Correct.** Many people hold an interdependent view of the self, in which identity and good decisions are rooted in relationships and family harmony, not only personal preference (Markus & Kitayama, 1991). The student states this value and does not want to bring shame on the family. Working within that frame, such as exploring paths that honor both art and family, respects the client as the expert on their own culture (NASW Code 1.05(c)). It beats B because a family meeting may help later, but the student has not asked for one.

**Takeaway:** Treat family-centered decision-making as part of the client's cultural view of self, not dependence; help clients choose within their own values.

**Sources:** Markus, H. R., & Kitayama, S. (1991). Culture and the self: Implications for cognition, emotion, and motivation. Psychological Review, 98(2), 224-253, whole article; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 76. ASM-1515 (rev 2) · Best answer: C

A 20-year-old Mexican American college student meets with a campus social worker. Through high school, the student tried to fit in with white classmates and avoided speaking Spanish. After a Chicano history course, the student now spends time only with Latino peers and distrusts white faculty. The student feels angry about years of 'trying to be someone else.' In the racial/cultural identity development model, which stage does this **MOST** likely reflect?

**A.** Conformity
**B.** Dissonance
**C.** Resistance and immersion
**D.** Introspection

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Conformity describes preference for dominant-culture values and devaluing of one's own group. It matches the student's high school years, but the current pattern of rejecting the dominant culture marks a later stage.

**B** **Incorrect** (trap: Wrong model or concept). Dissonance is the period of confusion when experiences challenge earlier conformity attitudes. The history course may have triggered it, but the student now firmly embraces their own group and rejects the dominant one, which goes beyond questioning.

**C** **Correct.** In the racial/cultural identity development model, the resistance and immersion stage brings strong endorsement of one's own group, rejection of dominant-culture values and anger at past oppression or self-denial (Sue et al., 2022). This student has moved from trying to fit in with white peers to associating only with Latino peers, distrusting white faculty and feeling angry about earlier self-denial. It beats B because dissonance is the earlier stage of confusion and conflict, when prior beliefs are questioned but not yet replaced by full commitment to one's own group.

**D** **Incorrect** (trap: Wrong model or concept). Introspection follows resistance and immersion: the person starts to question rigid group loyalty and the energy spent opposing the dominant culture, often feeling torn. This student's exclusive same-group

ties, blanket distrust and anger show the earlier stage is still in force, not yet being questioned (Sue et al., 2022).

**Takeaway:** In racial/cultural identity development, resistance and immersion means embracing one's own group and rejecting the dominant culture, often with anger.

**Sources:** Sue, D. W., Sue, D., Neville, H. A., & Smith, L. (2022). Counseling the Culturally Diverse: Theory and Practice (9th ed.). Wiley, Ch. 6: racial/cultural identity development model

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.19 Racial, ethnic, cultural, spiritual development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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## 77. INT-0007 (rev 1) · Best answer: D

A social worker records a client's daily anxiety ratings for 2 weeks, then begins cognitive behavioral therapy. Ratings fall steadily over the next 6 weeks. During the same period, the client starts a new job they enjoy. Which conclusion is **BEST** supported by these data?

**A.** The cognitive therapy caused the drop in the client's anxiety

**B.** The new job, rather than therapy, caused the change

**C.** The data are unusable because the baseline was too short

**D.** Anxiety improved, but the design cannot isolate the cause

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). AB designs cannot establish causation. With a new job starting during the intervention phase, history is a plausible rival explanation, so a causal claim about therapy goes beyond the data.

**B** **Incorrect** (trap: Wrong model or concept). This makes the same error in the other direction. The design can no more show that the job caused the change than it can show that therapy did.

**C** **Incorrect** (trap: Wrong model or concept). Fourteen daily ratings give a reasonable baseline, since several stable points are generally enough. The data are usable; the limit is causal inference, not baseline length.

**D** **Correct.** This is an AB single-system design: a baseline phase followed by an intervention phase. It can show that change followed the start of the intervention, but it cannot rule out other explanations such as history, here a new job starting at the same time (Bloom, Fischer & Orme, 2009). The supported conclusion is that anxiety improved and the cause is uncertain. It beats A because claiming therapy caused the change ignores the competing event, a threat to internal validity that an AB design cannot control.

**Takeaway:** An AB design can show change after intervention but cannot prove the intervention caused it; outside events remain rival explanations.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, AB design; threats to internal validity

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 78. INT-1164 (rev 2) · Best answer: C

During a video couples session, one partner opens a talk about chores by saying, 'You never do anything around here unless I nag you. You only think about yourself.' The other partner sits back and calmly starts to explain their week. The social worker uses the Gottman Method. What is the **BEST** intervention?

**A.** Ask the listening partner to repeat back what they heard
**B.** Suggest a 20-minute break so both partners can calm down
**C.** Coach the speaker to restate the complaint as a feeling and need
**D.** Explore how chores were divided in each partner's childhood home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Reflective listening helps once a complaint is stated without blame. Having the partner mirror a character attack keeps the criticism at the center and invites defensiveness; restating comes first.

**B** **Incorrect** (trap: Misuses the technique). A self-soothing break of about 20 minutes is the Gottman response to flooding. The listener is calm and engaged, so stopping now interrupts a moment the social worker can use.

**C** **Correct.** Gottman names criticism, an attack on a partner's character, as one of the Four Horsemen. Its antidote is a gentle start-up, in which the speaker describes their own feeling and states a positive need without blame (Gottman & Silver, 2015). 'You never' and 'you only think about yourself' attack character instead of naming a specific complaint, and there is no mockery or name-calling (marks of contempt). This is criticism, so coaching the speaker to restate the complaint changes the pattern at its source. It beats A because having the partner repeat back an attack rehearses the criticism instead of changing it.

**D** **Incorrect** (trap: Wrong model or concept). Family-of-origin patterns are a Bowen-style focus and may matter later. The Gottman approach first changes how the couple talks to each other in the moment.

**Takeaway:** In the Gottman Method, the antidote to criticism is a gentle start-up: describe your own feeling and state a positive need, without blame.

**Sources:** Gottman, J. M., & Silver, N. (2015). The Seven Principles for Making Marriage Work (rev. ed.). Harmony, Four Horsemen; soften startup

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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### 79. ETH-0100 (rev 1) · Best answer: C

A social worker in private practice receives a subpoena from the attorney for a client's former partner in a custody case. It asks for the client's full treatment record. The client has not signed a release, and no judge has ordered the records. What should the social worker do **FIRST**?

**A.** Send the full record to the attorney by the stated deadline
**B.** Disregard the subpoena since it lacks a judge's signature
**C.** Contact the client to discuss the subpoena and their wishes
**D.** Send the attorney a letter asserting privilege for the client

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Misreads law or ethics). Releasing the whole file on an attorney's subpoena treats a request as if it were a court order. Without the client's authorization or a judge's order, this breaches confidentiality and privilege. Even with an order, only what is required is disclosed.
- **B** **Incorrect** (trap: Misreads law or ethics). A subpoena is a legal document that needs a timely response, even without a judge's signature. Ignoring it can bring legal penalties. The right course is to respond through proper steps, neither disregarding it nor complying automatically.
- **C** **Correct.** A subpoena from an attorney is not a court order, and the privilege belongs to the client, not the social worker. Records go out only with the client's written authorization or a court order (NASW Code 1.07(b), 1.07(j)). Contacting the client first tells them about the request and lets them decide whether to authorize release or have their own attorney object. It beats D because asserting privilege is the right move once the client declines to authorize release; the client, as holder, decides first. HIPAA likewise bars release on a bare subpoena without notice or a protective order (45 CFR 164.512(e)(1)(ii)).
- **D** **Incorrect** (trap: Right action, wrong time). Asserting privilege on the client's behalf is appropriate when the client does not authorize release. Doing it before speaking with the client skips the holder's decision, since the client may choose to release the records or have counsel object.

**Takeaway:** A subpoena is not a court order: notify the client first, and release records only with written authorization or a judge's order.

**Sources:** NASW Code of Ethics (2021), 1.07(j); NASW Code of Ethics (2021), 1.07(b); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(ii)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 80. ETH-1604 (rev 1) · Best answer: A

A home health social worker visits an older client who speaks Vietnamese to set up a schedule for home-delivered meals. The client's adult child offers to interpret. With the child out of the room, the social worker uses a telephone interpreter to explain that a free interpreter is available. The client clearly prefers the child, and the child agrees. What should the social worker do **NEXT**?

- **A.** Proceed as the client chose and document the choice
- **B.** Keep the telephone interpreter on the line for the visit
- **C.** Explain again why a trained interpreter is better

**Why each option is right or wrong**

**A** **Correct.** Federal rules for health programs allow an accompanying adult to interpret when the client, told privately through a qualified interpreter, asks for it, the adult agrees, the choice is documented and reliance suits the situation (45 CFR 92.201(e)(2)). Each condition is met, and the topic is a routine meal schedule rather than sensitive clinical content. It beats B because insisting on the telephone interpreter overrides an informed, low-risk choice by a competent client (NASW Code 1.02). A qualified interpreter would be the better choice for sensitive or complex topics.

**B** **Incorrect** (trap: Overrides the client's choice). A qualified interpreter is the default and would be right if the client had not made an informed choice or the topic were sensitive. Here, overriding a competent client's private, informed preference on a routine matter is paternalistic.

**C** **Incorrect** (trap: Right action, wrong time). The client has already heard, privately and through a qualified interpreter, that a free interpreter is available. Repeating the case for it pressures the client and suggests the choice is not respected.

**Takeaway:** A competent client may choose an adult companion to interpret after a private offer of a free qualified interpreter, when the topic suits it.

**Sources:** Section 1557 of the Affordable Care Act regulations (45 CFR Part 92), 45 CFR 92.201(e)(2); NASW Code of Ethics (2021), 1.02

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 81. ETH-1089 (rev 2) · Best answer: D

When values in the NASW Code of Ethics conflict, the Code asks social workers to use their informed judgment. According to the Code, how else should such a decision be evaluated?

**A.** By whether the client prefers the outcome
**B.** By the employing agency's written policies
**C.** By the Ethical Principles Screen's ranking
**D.** By how a peer review process would view it

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Client preference carries great weight under self-determination, but it is not the measure the Code names for judging whether a professional resolved an ethical conflict well.

**B** **Incorrect** (trap: Misreads law or ethics). Agency policy is one consideration, but the Code says employer policies should not interfere with ethical practice (3.09(d)). It is not the standard the Code names for judging decisions.

**C** **Incorrect** (trap: Wrong model or concept). The screen ranks principles for hard cases, but it comes from an ethics textbook, not from the NASW Code, which deliberately declines to rank its values.

**D** **Correct.** The Code's Purpose section explains that it does not rank its values, principles or standards, and that reasonable social workers can disagree about priorities. Decisions rest on the worker's informed judgment and should also consider how the issues would be judged in a peer review process applying the profession's

standards (NASW Code, Purpose). D beats C because the Ethical Principles Screen is a useful textbook tool, not part of the Code, and the Code itself sets no ranking.

**Takeaway:** The Code does not rank its values; resolutions should be defensible to a peer review applying professional standards.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 82. INT-1005 (rev 1) · Best answer: A

A client with PTSD after a car crash meets with a social worker trained in cognitive processing therapy (CPT) but not in EMDR. After hearing about both treatments, the client declines CPT, saying that writing about the crash feels unbearable, and asks for EMDR. Another practice in the area offers EMDR and accepts the client's insurance. What is the **MOST** appropriate action?

**A.** Refer the client elsewhere for EMDR
**B.** Begin EMDR after studying a published protocol manual
**C.** Encourage the client to try a few CPT sessions first

**Why each option is right or wrong**

**A** **Correct.** Social workers practice within their competence and refer when another professional's specialized skills are needed to serve the client fully (NASW Code 1.04(a), 1.16(a)). EMDR, like CPT, is a recommended trauma-focused therapy for PTSD (VA/DoD, 2023). The client made an informed choice after hearing both options, and a trained provider is available nearby. It beats C because pressing the treatment the worker happens to offer overrides an informed, reasonable preference and risks dropout.

**B** **Incorrect** (trap: Outside the social work role). EMDR requires formal training with consultation; reading a manual does not make the worker competent. Using an approach new to the worker without training, consultation, and supervision conflicts with the competence standard (NASW Code 1.04(b)).

**C** **Incorrect** (trap: Overrides the client's choice). CPT is equally well supported, and the worker may describe its benefits. Pushing it after the client has declined disregards an informed choice. Encouragement would fit if the client were undecided or no EMDR provider were available.

**Takeaway:** When a client makes an informed choice of an evidence-based treatment outside the worker's training, refer to a trained provider.

**Sources:** NASW Code of Ethics (2021), 1.04(a)-(b); NASW Code of Ethics (2021), 1.16(a); VA/DoD Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder (2023), recommended trauma-focused psychotherapies
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 83. INT-1632 (rev 1) · Best answer: B

A client at a community agency brings an eviction summons that requires a written response to the court within 5 days. The social worker listens as the client describes fear of losing the apartment. The client says the landlord's claims are false and asks what to write in the response. What should the social worker do **NEXT**?

**A.** Ask how the client is coping with the threat of eviction
**B.** Connect the client today with a legal aid office
**C.** Help the client draft a reply disputing the landlord's claims
**D.** Help the client list other places to live if evicted

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The social worker has already listened to the client's fear. With a court deadline days away, more exploration delays the step that protects the client's housing.

**B** **Correct.** In problem solving, part of the social worker's role is recognizing when a problem needs another profession's expertise and making a timely link. Deciding what to argue in a court filing is legal advice, outside social work competence (NASW Code 1.04(a)), and the Code directs referral when specialized knowledge is needed (1.16(a)). The 5-day deadline makes today the time to act. It beats C because helping write the court response would put the social worker in the role of legal adviser, and an error could cost the client the case.

**C** **Incorrect** (trap: Outside the social work role). Helping draft a legal reply feels supportive, but choosing what to argue and how to file is legal work. The social worker can help the client gather documents for an attorney, not shape the court filing.

**D** **Incorrect** (trap: Good but not best). A backup housing plan may be needed later, but the client wants to contest claims believed to be false. Planning a move first treats the eviction as settled and uses time the legal response requires.

**Takeaway:** When a client's problem requires legal expertise, the social worker's role is a timely link to legal help, not drafting legal arguments.

**Sources:** NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.16(a)
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.4 What the social worker does in problem solving · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 84. ASM-1013 (rev 2) · Best answer: A

Which term describes the combination of mutism, holding odd postures for long periods, resisting being moved and repeating other people's words?

**A.** Catatonia
**B.** Acute dystonia
**C.** Negative symptoms

**Why each option is right or wrong**

**A** **Correct.** Catatonia is identified when at least 3 of 12 motor and behavioral features are present, including mutism, posturing, negativism (resisting instructions or movement) and echolalia (DSM-5-TR). The description includes all four. Catatonia can occur with psychotic, mood or medical conditions and can become life-threatening, so it calls for prompt medical attention. It beats C because negative symptoms such as reduced speech do not include holding odd postures, actively resisting movement or echoing others' words.

**B** **Incorrect** (trap: Near-miss diagnosis). Acute dystonia is a sudden, often painful muscle spasm, usually of the neck, jaw or eyes, soon after starting or raising an antipsychotic. It does not explain mutism, resistance to repositioning or echoed speech.

**C** **Incorrect** (trap: Near-miss diagnosis). Negative symptoms include reduced speech and expression, which can look like mutism. They do not include held postures, active resistance to being moved or repeating another person's words.

**Takeaway:** Mutism, posturing, negativism and echolalia together signal catatonia, a potentially dangerous syndrome that needs prompt medical evaluation.

**Sources:** DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Catatonia; DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Medication-Induced Acute Dystonia
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 85. ETH-1063 (rev 1) · Best answer: C

During a video session, a client tearfully discusses the anniversary of a sibling's death and says, 'Sometimes I wonder what the point is.' Before the social worker can ask more, the video freezes and the call drops. Reconnecting fails. At intake, they agreed the social worker would phone the client if video failed. What should the social worker do **FIRST**?

**A.** Ask local police to check on the client at home
**B.** Send a secure message offering to finish next week
**C.** Call the client's number on file

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). A welfare check fits imminent risk or a failed attempt to reach a client after a high-risk statement. The worker has not yet tried the agreed call or assessed the remark, so this escalates too soon.

**B** **Incorrect** (trap: Ignores risk). Waiting a week leaves a possible suicidal statement unassessed and ignores the backup plan. Rescheduling fits a routine outage, not one that interrupts a risk disclosure.

**C** **Correct.** Social workers plan for technology failures and discuss backup procedures with clients early (Tech Standards 2.12), and they make reasonable efforts to maintain services when communication is disrupted (NASW Code 1.15). The agreed backup is a phone call, which also lets the worker finish assessing a possible suicidal statement. C beats A because the remark is ambiguous and no intent, plan or means is known;

emergency responders become appropriate if the client cannot be reached or the assessment shows imminent risk.

**Takeaway:** When video fails, use the agreed backup channel first, especially if a risk disclosure was interrupted; escalate if contact fails.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.12; NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 86. ETH-1044 (rev 2) · Best answer: B

In a caregiver support program, a client caring for a parent with major neurocognitive disorder admits shoving the parent twice this week when the parent resisted bathing. The parent has bruises on both upper arms. The client is tearful and asks for help managing stress. What should the social worker do **FIRST**?

**A.** Arrange respite care to ease the caregiver's strain
**B.** Contact adult protective services about the incidents
**C.** Explore the client's guilt about the shoving
**D.** Teach the client calmer ways to handle bathing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Respite addresses the stress behind the shoving and belongs in the plan. As a first step it treats repeated physical harm to a dependent adult as a caregiver-support problem and leaves the report undone.

**B** **Correct.** Physical abuse of a dependent adult, with injuries, is reportable to adult protective services, which most state laws require. Legal duties can outweigh loyalty to the client, who should be told about the report (NASW Code 1.01), and the disclosure is limited to what the report needs (NASW Code 1.07(c)). B beats A because respite is an important part of the response, but it does not replace the report or the protective assessment adult protective services provides.

**C** **Incorrect** (trap: Right action, wrong time). Exploring the client's guilt can support the caregiver and the relationship later. With repeated physical harm and bruises already disclosed, the protective report comes before emotional processing.

**D** **Incorrect** (trap: Good but not best). Skills for handling care resistance can reduce future incidents and belong in the plan. Teaching them first assumes the risk is managed and delays the report the injuries call for.

**Takeaway:** Disclosed physical abuse of a dependent adult calls for a protective report first; caregiver support follows and does not replace it.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 1.01
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 87. ASM-1055 (rev 1) · Best answer: C

A school social worker meets a quiet 10-year-old whose parent drinks heavily. Teachers describe the student as 'no trouble at all.' The student spends recess alone drawing, rarely asks for help and is often forgotten when groups are formed. At home, the student stays in their room for hours. Which family role in alcohol-affected families does this pattern **MOST** likely reflect?

**A.** Family hero
**B.** Scapegoat
**C.** Lost child
**D.** Mascot

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). The family hero is the high achiever who takes on responsibility and earns praise to make the family look good. Being undemanding resembles the hero's good behavior, but this student seeks no achievement or recognition.

**B** **Incorrect** (trap: Wrong model or concept). The scapegoat acts out and draws blame and negative attention away from the parent's drinking. This student avoids attention entirely, the opposite of the scapegoat's defiance.

**C** **Correct.** Wegscheider-Cruse described roles children may adopt in families affected by addiction. The lost child copes by withdrawing: quiet, solitary, undemanding and easily overlooked, which matches this student's isolation and invisibility in groups. It beats A, the family hero, because the hero seeks achievement and approval that bring the family credit, whereas this student draws no attention at all. The roles are descriptive, not diagnoses, but they help workers notice children whose distress is easy to miss (Wegscheider-Cruse, 1989).

**D** **Incorrect** (trap: Wrong model or concept). The mascot uses humor and clowning to ease family tension and draw attention. This student is quiet and solitary rather than playful or attention-seeking.

**Takeaway:** In addiction-affected families, a withdrawn, undemanding, overlooked child fits the lost child role, whose distress is easily missed.

**Sources:** Wegscheider-Cruse, S. (1989). Another Chance: Hope and Health for the Alcoholic Family (2nd ed.). Science and Behavior Books, family roles

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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### 88. INT-1518 (rev 2) · Best answer: B

A client who sought telehealth counseling for work stress has canceled 2 of the last 3 sessions by text, each time citing a busy schedule. The agency discharges clients after 3 missed sessions. What should the social worker do **FIRST**?

**A.** Send a letter explaining the agency's discharge policy
**B.** Reach out to discuss what is making sessions hard to keep
**C.** Wait to see whether the client attends the next scheduled session

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Clients should know the attendance policy, but a letter sent before any conversation feels punitive and may push an ambivalent client away. The policy can be explained during a discussion of barriers.

**B** **Correct.** Missed sessions often signal ambivalence or practical barriers, and voluntary clients stay engaged when the worker raises the pattern openly and without blame. Asking what is making sessions hard, such as timing, fit, or doubts about the work, can lead to schedule changes or renewed goals and guards against ending care for a client who still needs it (NASW Code 1.17(b); Hepworth et al.). It beats A because the policy can be covered in that conversation; a letter alone reads as a warning.

**C** **Incorrect** (trap: Good but not best). Waiting respects the client's pace, but two cancellations in three weeks are already a pattern. Staying silent risks a third miss and automatic discharge before the barrier is ever discussed. Waiting fits after a single, clearly explained cancellation.

**Takeaway:** When a voluntary client starts missing sessions, explore barriers and ambivalence with the client before warnings or discharge.

**Sources:** NASW Code of Ethics (2021), 1.17(b); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Managing barriers to change

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 89. ASM-1544 (rev 2) · Best answer: B

A 51-year-old client was released 3 weeks ago after 14 years in prison and now lives with a sibling. The sibling says the client waits to be told when to eat or shower, freezes in the grocery store when faced with choices, and sits facing the door in every room. The client says they are glad to be home. At last week's visit, both learned that these are common adaptations to long incarceration. What should the social worker do **NEXT**?

**A.** Ask the sibling to set the daily schedule and make choices for now

**B.** Help the client practice making small daily choices with sibling support

**C.** Refer the client for an evaluation for major depressive disorder

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). A predictable routine can ease the first weeks home, but having the sibling make the client's choices recreates prison-style control. It maintains the dependence on outside structure that the client needs to unlearn.

**B** **Correct.** Long incarceration can produce institutionalization: dependence on outside structure, a weakened capacity to make decisions and start activities, and hypervigilance (Haney, 2001). These are learned adaptations, and Haney calls for transitional help to readapt to life outside. After normalizing the reactions, the next step is graded practice in everyday decisions, such as choosing a meal, with the sibling encouraging rather than deciding. It beats A because taking over the client's choices would recreate the external control that maintains the dependence.

**C** **Incorrect** (trap: Skips assessment). Withdrawal after release can resemble depression and should be assessed if mood symptoms appear. Here the client reports being glad to

be home, and the specific pattern of waiting for permission and freezing at choices points to institutional adaptation.

**Takeaway:** Institutionalization after long incarceration involves dependence on external structure; rebuild decision-making through graded everyday choices, not continued outside control.

**Sources:** Haney, C. (2001). The psychological impact of incarceration: Implications for post-prison adjustment. U.S. Department of Health and Human Services, ASPE, From Prison to Home conference, institutionalization effects; transitional services

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.13 Effects of out-of-home placement (hospital, foster, residential, justice) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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## 90. INT-1516 (rev 1) · Best answer: A

A physician asks a hospital social worker to assess home safety for a 79-year-old patient admitted after a fall. When the social worker arrives, the patient says, 'I didn't ask for a social worker. I'm going home, period.' The patient is alert and oriented. What should the social worker do **FIRST**?

**A.** Explain that the visit aims to help the patient get home safely

**B.** Leave and document that the patient declined social work services

**C.** Begin the home safety questions the physician requested

**Why each option is right or wrong**

**A** **Correct.** Nonvoluntary clients engage best when the worker clarifies the purpose of contact and ties it to a goal the client already holds (Rooney, 2009). The patient's goal is going home, and the referral exists to make that goal workable, so explaining this reframes the visit as help with the patient's own aim. It beats B because a competent patient may decline, but a refusal given before the patient knows why the worker came is not yet an informed choice; once the purpose is clear, the decision is respected.

**B** **Incorrect** (trap: Good but not best). Respecting refusal is right for a competent adult who understands what is offered. Leaving now, before explaining the purpose, may forfeit help with the very goal the patient wants.

**C** **Incorrect** (trap: Right action, wrong time). The home safety assessment is the eventual task, but starting it over the patient's objection ignores the refusal and invites pushback. Engagement and consent come first.

**Takeaway:** With nonvoluntary clients, first clarify the purpose of contact and link it to a goal the client already holds before assessing.

**Sources:** Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, engaging nonvoluntary clients: clarifying purpose and client goals

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 91. ASM-1605 (rev 1) · Best answer: A

A child welfare social worker meets with a parent whose 4-year-old was placed in foster care after being left alone overnight. The court-approved case plan lists nine tasks, including parenting classes, a substance use assessment, housing applications and a job search. The parent says, 'I can't do all of this at once, and I don't know where to start.' The review hearing is in 5 months. What should the social worker do **FIRST**?

**A.** Rank the tasks with the parent by their link to safety
**B.** Help the parent enroll in the nine services this week
**C.** Ask the court to reduce the number of required tasks
**D.** Refer the parent to counseling for feeling overwhelmed

**Why each option is right or wrong**

**A** **Correct.** When a plan feels unmanageable, partializing breaks it into ordered steps, and priorities follow the concern that brought the family to the agency (Hepworth et al.). The child was left alone overnight, so the tasks most directly tied to safe supervision come first, with others sequenced over the 5 months. Ranking the tasks together also builds the parent's ownership of the plan (NASW Code 1.02). It beats C because asking the court to cut tasks comes after the social worker and parent have tested whether a sequenced plan is workable.

**B** **Incorrect** (trap: Good but not best). Starting everything at once confirms the parent's fear of being overwhelmed and invites missed appointments that can look like noncompliance to the court. Services can be staggered across the 5 months before the hearing.

**C** **Incorrect** (trap: Right action, wrong time). Advocating to change the plan is appropriate if, after sequencing, the tasks still cannot be done in time. Going to the court first skips the work of prioritizing with the parent and may prove unnecessary.

**D** **Incorrect** (trap: Hands off too soon). Feeling overwhelmed by nine tasks is an expected reaction to an unordered plan, not a separate problem that needs referral. Adding another service lengthens the list before the social worker has helped organize it.

**Takeaway:** Partialize an overwhelming service plan with the client, ordering tasks by their link to the safety concern before seeking plan changes.

**Sources:** Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, partializing problems and prioritizing goals; NASW Code of Ethics (2021), 1.02
**Area** II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 92. ASM-1571 (rev 1) · Best answer: C

A client joins a video session 20 minutes late for the third week in a row. The client apologizes and says, 'I know, I know.' The social worker wants to understand what is getting in the way without sounding critical. Which question is **BEST**?

**A.** Why do you keep joining late?
**B.** Is it your work schedule, or are you unsure about coming?
**C.** What has been making it hard to join on time?

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). This asks about the right topic in the wrong form. 'Why' questions often sound like demands for justification, and a client who is already apologizing may become defensive rather than open.

**B** **Incorrect** (trap: Misuses the technique). Combining two options in one question limits the answer to the worker's guesses and hints at reluctance. An open question lets the client name the barrier, which may be neither choice.

**C** **Correct.** Open 'what' questions invite clients to explain in their own terms, while 'why' questions tend to sound accusatory and prompt defensiveness (Hepworth et al., counterproductive communication patterns). The client is already apologizing, so a nonjudgmental open question is most likely to surface the real obstacle, whether scheduling, technology or mixed feelings. It beats B because offering two choices narrows the answer and implies the client may not want to come.

**Takeaway:** Use open 'what' or 'how' questions to explore problems; avoid 'why' questions and double or leading questions that sound critical.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding counterproductive communication patterns: 'why' and leading questions
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 93. ETH-1120 (rev 1) · Best answer: C

After staff cuts, social workers on a hospital's medical units have handled nearly double their usual discharges for a year. A social worker notices that several colleagues, like the worker, feel drained, have grown detached from patients, and doubt their work makes a difference. Management's response so far has been an optional lunchtime wellness webinar. What is the **BEST** action for the social worker?

**A.** Encourage colleagues to attend the wellness webinars
**B.** Build a personal routine of exercise, sleep and breaks
**C.** Propose workload relief to leaders using discharge data

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Wellness programs can help, but they treat a workload problem as an individual coping problem. Promoting them alone leaves the main driver of the team's exhaustion untouched.

**B** **Incorrect** (trap: Good but not best). Healthy routines support resilience and are worth keeping. When a whole team shows the same signs after a workload surge, though, individual coping cannot fix a cause that sits in the organization.

**C** **Correct.** Burnout, marked by exhaustion, detachment and a reduced sense of accomplishment, arises mainly from chronic workplace stressors such as excessive workload (Maslach & Leiter, 2016). Here the pattern is shared across the team and tied to a year of doubled discharges, so the cause is organizational. Social workers work to improve agency policies and services, and administrators should remove conditions that interfere with ethical practice (NASW Code 3.09(b), 3.07(d)). C beats B

because personal self-care helps one person but leaves in place the workload driving the team's burnout.

**Takeaway:** Burnout driven by workload calls for organizational change through agency channels, not only individual self-care.

**Sources:** Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: recent research and its implications for psychiatry. World Psychiatry, 15(2), 103-111, burnout dimensions and workplace causes; NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 3.07(d)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 94. INT-1557 (rev 1) · Best answer: C

In an in-home parenting program, a parent agreed to ignore their 4-year-old's whining for snacks and to praise polite requests. Four days later, the parent reports that the whining has become louder and longer and asks whether the plan is failing. The child is otherwise healthy and safe. What should the social worker do **NEXT**?

**A.** Explore with the parent how the louder whining is affecting them
**B.** Replace planned ignoring with a short time-out for whining
**C.** Explain the expected surge and coach continued consistency

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The parent's stress deserves attention, but the parent asked a direct question and the stem already shows what is happening. Exploring feelings first leaves the parent without the information needed to stay on the plan tonight.

**B** **Incorrect** (trap: Misuses the technique). Time-out can be useful, but switching tactics during a surge breaks the extinction process and teaches the child that louder whining brings change. Time-out fits behaviors that cannot safely be ignored, such as hitting.

**C** **Correct.** When a behavior's reinforcer is withheld, the behavior often gets worse before it fades, a pattern called an extinction burst. Louder, longer whining after 4 days is that expected surge, not a failed plan. Explaining it and coaching consistency, while continuing praise for polite requests, keeps the parent from giving in at the peak, which would teach the child that escalation works (Kazdin, 2005). It beats B because switching tactics now breaks the extinction process at the hardest moment.

**Takeaway:** A temporary increase after a reinforcer is withdrawn is an expected extinction burst; coach parents to stay consistent rather than change plans.

**Sources:** Kazdin, A. E. (2005). Parent Management Training: Treatment for Oppositional, Aggressive, and Antisocial Behavior in Children and Adolescents. Oxford University Press, extinction and extinction bursts
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 95. ASM-1656 (rev 2) · Best answer: B

A 15-year-old who uses a wheelchair has few accessible activities nearby and spends most evenings in an online group for teens with the same condition. The teen calls these group members their closest friends. After a news story about online predators, the parent asks the social worker to support removing the teen's internet access. What is the **MOST** appropriate response?

**A.** Support the parent's plan until the teen is older
**B.** Review with the teen and parent the group's benefits and risks
**C.** Tell the parent the group is a strength that should continue
**D.** Suggest an in-person activity to replace the online group

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Removing access may feel protective, but it cuts the teen off from their main peer support and ignores the teen's view. Limits might fit later if assessment finds real danger, such as an adult seeking private contact.

**B** **Correct.** Technology brings both benefits and risks, so assessment should weigh both before anyone acts. The U.S. Surgeon General's 2023 advisory notes that social media can give youth positive community with others who share identities, abilities and interests, and it also names risks such as harassment and predatory contact. For a teen with few accessible local options, this group may be a key support. Exploring with the teen and parent what the group provides and what safety habits the teen uses lets them build a plan together. It beats C because endorsing the group before learning how it works dismisses the parent's real concern and skips the safety review.

**C** **Incorrect** (trap: Too directive). The group may well be protective, but declaring so before learning how the teen uses it skips assessment and brushes aside the parent's fear. A safety review comes before advising either way.

**D** **Incorrect** (trap: Good but not best). In-person activities are worth exploring, but few are accessible to this teen, and a replacement assumes the online friendships have no value. Adding options can come after the group's role is understood.

**Takeaway:** Assess both the supportive value and the risks of a client's online connections before endorsing or restricting them.

**Sources:** U.S. Surgeon General (2023). Social Media and Youth Mental Health: The U.S. Surgeon General's Advisory. U.S. Department of Health and Human Services, potential benefits and potential risks of social media **Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.21 Technology and social media effects on clients · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 96. ETH-1142 (rev 1) · Best answer: C

During a video session, a 15-year-old client says they want to die. When asked directly, the teen says they plan to take a parent's sleeping pills tonight and knows where they are kept. The teen begs the social worker not to tell anyone. A parent is at home in another room. What should the social worker do **NEXT**?

**A.** Complete a safety plan with the teen before involving anyone else
**B.** Have emergency responders sent before telling the parent
**C.** Bring the parent into the session to address safety now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). A safety plan matters, but it cannot hold an immediate plan with known means. The teen's request for secrecy does not outweigh imminent risk, and planning alone leaves the pills within reach tonight.

**B** **Incorrect** (trap: Good but not best). Emergency responders are the right call if no adult in the home can act or the teen leaves the session. Here a parent is in the home and can secure the pills and arrange evaluation faster and with less disruption.

**C** **Correct.** Confidentiality gives way when disclosure is needed to prevent serious, foreseeable and imminent harm to the client (NASW Code 1.07(c)), and HIPAA permits disclosure to a person able to lessen the threat, including family (45 CFR 164.512(j)(1)(i)). The teen has a plan for tonight and knows where the pills are, and a parent who can remove them and get the teen evaluated is steps away. The worker tells the teen first, as feasible (1.07(d)). C beats B because the parent can act at once while the teen stays connected; responders are right if the parent cannot be reached or the teen ends the call.

**Takeaway:** Imminent self-harm risk overrides a minor's request for secrecy; involve the nearest adult who can remove the means and get care.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 1.07(d); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 97. ETH-1559 (rev 1) · Best answer: A

A social worker chairs the quality committee of a community crisis service. A chart audit shows that Black youth receive police-involved crisis responses about twice as often as white youth with similar risk ratings. Staff say each decision followed protocol. What is the **BEST** response by the committee chair?

**A.** Lead a review of the dispatch process with staff and youth families

**B.** Require every staff member to complete implicit bias training

**C.** Gather another year of data before drawing any conclusions

**D.** Remind staff that the protocol applies equally to every youth

**Why each option is right or wrong**

**A** **Correct.** Anti-racist practice examines how institutional routines, not only individual attitudes, produce unequal outcomes. The Code calls on social workers to act against racism and to hold institutions accountable (NASW Code 1.05(b), 1.05(c); 6.04(d)). A gap that persists when every decision followed protocol points to the process itself. Reviewing dispatch criteria with staff and the families affected shares power and targets the source. It beats B because bias training addresses individual attitudes but leaves an unexamined process in place.

**B** **Incorrect** (trap: Good but not best). Implicit bias training can help individual staff and may be part of the plan. By itself it treats the gap as a matter of personal attitudes and leaves the dispatch criteria and routines that produced it unexamined.

**C** **Incorrect** (trap: Ignores risk). The audit already compares youth with similar risk ratings and shows a large gap. Waiting a year lets police-involved responses continue unequally; ongoing monitoring belongs alongside changes, not in place of them.

**D** **Incorrect** (trap: Wrong model or concept). Stressing equal application assumes that a neutral rule produces fair results. Here a protocol applied as written is yielding unequal outcomes, which is exactly what an anti-racist review must examine.

**Takeaway:** When a neutral-seeming process yields racial disparities, examine and change the process with those affected, not only individual attitudes.

**Sources:** NASW Code of Ethics (2021), 1.05(b); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 98. INT-1678 (rev 1) · Best answer: B

A reentry program is writing outcome objectives for a new grant. A draft reads: 'Within 12 months, increase stable housing among participants by 30%.' Staff have no figures on how many people like these participants are stably housed now. What should the social worker do **FIRST**?

**A.** Lower the target to 10% so it is achievable

**B.** Find the current housing rate for comparable participants

**C.** Reword it to promise better housing stability for participants

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Adjusting the target may be right once the starting point is known. Picking 10% now is as arbitrary as 30%, because the program still has nothing to measure change against.

**B** **Correct.** An outcome objective needs a measurable criterion, and a percentage change means little without a starting point. Getting baseline data on housing among similar participants lets staff set a target that is realistic and lets the evaluation show whether change happened. It beats A because lowering the number is still a guess; a 10% target could be trivial or out of reach until the current rate is known (Kettner, Moroney & Martin, 2022; Doran, 1981).

**C** **Incorrect** (trap: Misuses the technique). This drops the measurable criterion, turning the objective into a broad goal. Goals can be general, but objectives state what will change, by how much and by when, so progress can be measured.

**Takeaway:** Set an objective's numeric target from baseline data; without a starting point, a promised percentage change is a guess.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities; Doran, G. T. (1981). There's a S.M.A.R.T. way to write management's goals and objectives. Management Review, 70(11), 35-36, whole article

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.2 Setting program objectives and outcomes · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 99. ETH-1067 (rev 1) · Best answer: A

A social worker leads a weekly video support group for adults with chronic pain. Members agreed to join from private spaces. Midway through, one member speaks up from a crowded coffee shop, with nearby voices clearly audible, just as another member begins describing a recent hospitalization. What should the social worker do **FIRST**?

**A.** Pause and ask the member to move somewhere quiet or wear headphones
**B.** Remind everyone of the group's confidentiality agreement and continue
**C.** Mute the member's microphone and continue the discussion

**Why each option is right or wrong**

**A** **Correct.** In group work, social workers seek agreement on each member's duty to protect what others share and explain that confidentiality cannot be guaranteed (NASW Code 1.07(f)). In a remote group, that duty includes who else can hear. Strangers can hear another member's disclosure through this member's speaker, so the session should pause before more is shared. A beats B because a general reminder lets the disclosure continue while bystanders can still hear it.

**B** **Incorrect** (trap: Good but not best). A reminder is useful, but continuing lets the next member's disclosure be overheard by strangers right now. The immediate exposure has to be stopped first.

**C** **Incorrect** (trap: Misuses the technique). Muting stops the member from speaking but not from hearing; people nearby can still hear other members through the member's speaker. The privacy risk remains.

**Takeaway:** In remote groups, stop and fix an immediate privacy exposure before members continue sharing.

**Sources:** NASW Code of Ethics (2021), 1.07(f)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 100. ASM-1591 (rev 2) · Best answer: C

A hospital patient had a below-knee amputation after a work injury. The patient jokes with staff about 'saving on socks,' asks detailed questions about prostheses and calls coworkers daily. The social worker's assessment finds that the patient sleeps well, eats meals, works hard in physical therapy and talks about returning to work in a new role. A nurse requests a psychiatric consult because the patient 'must be in denial.' What should the social worker do **NEXT**?

**A.** Request the psychiatric consult the nurse suggested
**B.** Encourage the patient to set humor aside and grieve
**C.** Share the coping findings with the treatment team
**D.** Explore what the patient's jokes may be covering up

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). A psychiatric consult fits when assessment shows a mental health concern, such as depression or impaired functioning. Here the assessment shows good sleep, appetite and engagement, so a consult would pathologize healthy coping.

**B** **Incorrect** (trap: Too directive). Pushing the patient to grieve in a particular way imposes the worker's view of adjustment. People cope with loss differently, and this patient is functioning well; the patient may grieve in their own time.

**C** **Correct.** Coping assessment looks at how a client is actually functioning. Humor is a mature, adaptive defense (Vaillant, 1977), and seeking information and planning a return to work are problem-focused coping (Lazarus & Folkman, 1984). Sleep, appetite and active rehabilitation confirm the patient is adapting. Sharing these findings helps the team support the patient rather than treat strengths as pathology. It beats D because the assessment is already complete; probing for hidden distress treats healthy humor as a symptom.

**D** **Incorrect** (trap: Good but not best). Gentle exploration suits a patient whose humor seems to mask distress. This assessment already found good functioning and realistic planning, so searching for a hidden problem assumes pathology the facts do not show.

**Takeaway:** Judge coping by functioning; humor and active problem solving after a loss can be adaptive, not denial.

**Sources:** Vaillant, G. E. (1977). Adaptation to Life. Little, Brown, mature defenses: humor; Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, problem-focused coping
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 101. ASM-1581 (rev 1) · Best answer: C

A school social worker receives a referral for a 5-year-old who arrived 2 months ago from a country where English is not spoken. At school the child says almost nothing, yet watches classmates closely, points to request items and joins games. The parent reports that the child chats easily at home in the family's first language. Which explanation is **MOST** likely?

**A.** Selective mutism linked to school anxiety
**B.** Language disorder affecting expression
**C.** Normal silent phase of learning a new language
**D.** Autism spectrum disorder with language delay

#### Why each option is right or wrong

**A** **Incorrect** (trap: Near-miss diagnosis). Speaking at home but not at school fits part of the selective mutism picture. However, the diagnosis does not apply when silence comes from not yet knowing the language spoken in that setting, which is the case for a child here only 2 months (DSM-5-TR).

**B** **Incorrect** (trap: Near-miss diagnosis). A true language disorder appears in every language a child speaks. This child talks easily at home in the first language, which points to a language difference rather than a disorder.

**C** **Correct.** Many young children learning a second language go through a nonverbal period, listening and communicating with gestures before speaking the new language (Tabors, 2008). This child arrived only 2 months ago, speaks fluently in the home language and uses pointing and play to communicate. Selective mutism (A) is the strongest competitor, but DSM-5-TR rules it out when the silence reflects limited knowledge of, or comfort with, the language used in that setting.

**D** **Incorrect** (trap: Near-miss diagnosis). Watching peers, pointing to request and joining games show intact social communication. Combined with fluent home-language speech, these behaviors do not fit autism spectrum disorder.

**Takeaway:** Silence in a new language is usually a normal nonverbal phase; check the home language before suspecting a disorder.

**Sources:** Tabors, P. O. (2008). One Child, Two Languages: A Guide for Early Childhood Educators of Children Learning English as a Second Language (2nd ed.). Paul H. Brookes, Developmental sequence: nonverbal period; DSM-5-TR (APA, 2022), Anxiety Disorders: Selective Mutism

**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.8 Evaluating a client's communication skills · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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## 102. INT-1177 (rev 2) · Best answer: D

A client's partner, a commercial fisher, went missing at sea 14 months ago, and no body was found. Relatives urge the client to hold a funeral and 'get closure.' The client says, 'If I bury an empty coffin, I'm giving up on them, but I can't keep waiting like this either.' What is the **MOST** appropriate intervention?

**A.** Encourage the memorial service so the client can reach closure

**B.** Refer the client for prolonged grief disorder treatment

**C.** Support the client's hope that the partner may be found

**D.** Help the client accept holding both hope and grief at once

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Rituals help many mourners, and the client may later choose one. Pressing for a funeral to reach closure assumes the loss is settled, which is exactly what the client cannot know; Boss warns that insisting on closure hinders coping with this kind of loss.

**B** **Incorrect** (trap: Near-miss diagnosis). In DSM-5-TR, prolonged grief disorder follows a confirmed death and involves intense, impairing grief well beyond expected norms. Here the death is unproven and the client's struggle is the uncertainty itself, which calls for ambiguous-loss work rather than a disorder-specific referral.

**C** **Incorrect** (trap: Good but not best). Supporting hope alone validates only one side of what the client said and leaves no room for the grief they also feel. Ambiguous-loss work holds both truths instead of siding with hope or with death.

**D** **Correct.** Boss's ambiguous loss framework holds that when a loved one is physically missing without proof of death, closure may never come, and pushing for it adds distress. The aim is tolerating the uncertainty through both/and thinking: the partner may be alive and may be dead, and the client can grieve while still hoping (Boss, 2006). The client already voices both sides. It beats A because a funeral pressed on the client as a route to closure forces a choice the facts do not support and leaves the client feeling disloyal.

**Takeaway:** With a loss that has no proof or end, help the person hold hope and grief together; pressing for closure adds distress.

**Sources:** Boss, P. (2006). Loss, Trauma, and Resilience: Therapeutic Work With Ambiguous Loss. W. W. Norton, Both/and thinking; tolerance for ambiguity; the myth of closure; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 103. ETH-1685 (rev 1) · Best answer: A

After a client complains about hearing staff speak Spanish in the break room, a clinic director issues a rule that staff must speak only English at all times on the premises, including breaks. Several bilingual staff tell a social worker on the clinic's equity committee that they feel targeted. What is the social worker's **MOST** appropriate action?

**A.** Urge the director to replace the rule with one tied to client needs
**B.** Remind bilingual staff to follow the director's rule during review
**C.** File a discrimination complaint with a federal agency right away

**Why each option is right or wrong**

**A** **Correct.** Social workers act to prevent and eliminate discrimination in their organization's employment practices and hold institutions accountable (NASW Code 3.09(e); 1.05(c)). A rule requiring English at all times, breaks included, burdens staff by national origin; federal guidelines presume such blanket rules discriminatory, while narrower rules need a business necessity (29 CFR 1606.7). Raising this with the director and proposing a rule tied to real client-service needs uses the proper internal channel. It beats C because an outside complaint skips a direct route that is likely to work.

**B** **Incorrect** (trap: Misreads law or ethics). Asking staff to comply keeps a likely discriminatory rule in force and places the social worker on its side. Social workers should not condone discrimination (NASW Code 4.02). Following a narrower, justified rule could come later.

**C** **Incorrect** (trap: Skips assessment). Staff may file outside complaints, and that route becomes appropriate if internal efforts fail. Filing at once, before raising the concern with the director, skips the internal channel the committee role offers.

**Takeaway:** Blanket workplace rules that burden staff by language or national origin call for internal advocacy to replace them with narrowly justified policies.

**Sources:** NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 4.02; EEOC Guidelines on Discrimination Because of National Origin, 29 CFR 1606.7
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 104. ETH-1519 (rev 1) · Best answer: A

After discussing the request with the hospice team, a social worker accepts a family's invitation to a patient's memorial service. The social worker sits near the back, offers brief condolences to the family and leaves before the reception. Which concept does the social worker's attendance **MOST** likely illustrate?

**A.** Boundary crossing
**B.** Boundary violation
**C.** Dual relationship
**D.** Countertransference

**Why each option is right or wrong**

**A** **Correct.** A boundary crossing is a departure from a strict professional role that is not harmful and may even help the client; a boundary violation is a departure that exploits or harms (Gutheil & Gabbard, 1993). Attending a memorial after consulting the team, briefly and in a professional manner, is a recognized crossing that can support the family's grief. It beats B because nothing here exploits the family or serves the worker's own needs; the planning and limits are what keep it a crossing (Reamer, 2020).

**B** **Incorrect** (trap: Wrong model or concept). A violation harms or exploits the client, for example by serving the worker's own needs. A brief, planned, team-approved appearance at a memorial does neither, so this label does not fit the facts given.

**C** **Incorrect** (trap: Wrong model or concept). A dual relationship means taking on a second role, such as friend, business partner or employer. Attending a service in a professional capacity, without forming a new relationship, stays within a single role.

**D** **Incorrect** (trap: Wrong model or concept). Countertransference is the worker's emotional reaction to a client. The stem describes a planned professional action, and nothing suggests the worker's own feelings drove the decision.

**Takeaway:** Boundary crossings are harmless or helpful departures from strict roles; violations exploit or harm the client.

**Sources:** Gutheil, T. G., & Gabbard, G. O. (1993). The concept of boundaries in clinical practice: Theoretical and risk-management dimensions. American Journal of Psychiatry, 150(2), 188-196, boundary crossings versus boundary violations; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings and violations

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 105. ETH-1098 (rev 1) · Best answer: A

A colleague at a community agency is contesting a disciplinary action by management. The colleague asks the social worker to collect signatures from the social worker's clients on a petition supporting the colleague. How should the social worker respond to the request?

**A.** Decline to involve clients in the dispute
**B.** Leave the petition in the waiting room without comment
**C.** Ask the clients who know the colleague well

**Why each option is right or wrong**

**A** **Correct.** Social workers should not exploit clients in disputes with colleagues or engage clients in inappropriate discussion of those conflicts (NASW Code 2.04(b)). Clients may feel unable to refuse a request connected to their own worker, and the dispute has nothing to do with their care. A beats B because a petition placed in the waiting room still draws clients into a staff conflict, only less visibly.

**B** **Incorrect** (trap: Boundary slip or collusion). Passive placement feels less coercive, but it still uses the agency's service space to recruit clients into a staff dispute, and clients may assume their worker expects them to sign.

**C** **Incorrect** (trap: Boundary slip or collusion). Limiting the request to clients who know the colleague does not change the problem. Those clients may feel the most pressure, and the dispute is still unrelated to their services.

**Takeaway:** Keep clients out of disputes between colleagues and their employers.

**Sources:** NASW Code of Ethics (2021), 2.04(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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## 106. ETH-1116 (rev 2) · Best answer: B

What is the main purpose of the continuing education that state boards require for social work license renewal?

**A.** Raising the profession's standing among other disciplines
**B.** Protecting the public by keeping practitioners competent
**C.** Preparing licensees for specialty certification

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Professional standing may rise when practitioners stay current, but licensing boards are government agencies whose mandate is public protection, not the profession's reputation.

**B** **Correct.** Licensing exists to protect the public, and continuing education requirements serve that goal by helping licensees maintain competence as knowledge and practice change. ASWB's continuing education approval program frames acceptable courses around supporting ongoing professional competence so social workers serve the public better (ASWB, 2023). The Code likewise expects social workers to pursue continuing education to stay proficient (NASW Code 4.01(b)). B beats A because any gain in professional standing is a side effect, not the regulatory purpose.

**C** **Incorrect** (trap: Wrong model or concept). Some courses count toward specialty credentials, but boards require continuing education of every licensee to maintain competence, whether or not they pursue certification.

**Takeaway:** Regulation, including continuing education for renewal, exists to protect the public by maintaining practitioner competence.

**Sources:** ASWB (2023). ACE Handbook (December 2023 revision), Course relevance criteria and course review checklist; NASW Code of Ethics (2021), 4.01(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 107. ETH-1542 (rev 3) · Best answer: C

A client with bipolar I disorder arrives at an outpatient session talking rapidly and says they slept 3 hours in two nights. The client plans to withdraw their entire retirement savings tomorrow to fund a business idea from a late-night video. On a call from the session, the prescriber judges that a visit first thing tomorrow morning is soon enough. What should the social worker do **NEXT**?

**A.** Accept the withdrawal as the client's own financial choice
**B.** Send the client for a same-day crisis evaluation
**C.** Ask the client to delay the withdrawal until after the appointment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Respecting financial choices is right when a client can weigh them. Here sleep loss, rapid speech and an impulsive plan to empty retirement savings raise real doubt about judgment, so simply accepting the plan risks lasting harm.

**B** **Incorrect** (trap: Skips assessment). A same-day crisis evaluation fits when mood symptoms bring danger to self or others, such as threats, psychosis or reckless acts with physical risk. Here the prescriber has already judged that the morning visit is soon enough for a financial risk, so this escalation overrides that judgment and the client. New signs of danger before then would call for it.

**C** **Correct.** Self-determination assumes the ability to weigh a decision, and symptomatic bipolar disorder can impair decision making (Appelbaum, 2007). The client shows signs of an emerging episode and plans an irreversible step that could happen before the prescriber visit. Asking the client to wait until after the visit keeps the decision in the client's hands while preventing harm that cannot be undone, which honors self-determination without ignoring risk (NASW Code 1.02). It beats B because the prescriber has already set the timing of the evaluation; the remaining gap is the withdrawal itself.

**Takeaway:** When symptoms may impair judgment, ask the client to postpone irreversible decisions rather than overriding or ignoring the choice.

**Sources:** NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, bipolar disorder and impaired decision making
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 108. ASM-1114 (rev 2) · Best answer: A

Eight months after a hurricane damaged their home, a family that had been coping well becomes overwhelmed. Since the storm, one parent lost a job, insurance disputes dragged on, and a child's asthma has worsened. In the Double ABCX model, these later events are part of which factor?

**A.** Pile-up (aA factor)
**B.** Family resources (bB factor)
**C.** Family perception (cC factor)
**D.** Adaptation (xX factor)

**Why each option is right or wrong**

**A** **Correct.** McCubbin and Patterson (1983) extended Hill's (1949) ABC-X model to describe what happens after the first crisis. The aA factor, pile-up, is the build-up of the original stressor's hardships, new stressors and normal transitions over time. The job loss, insurance disputes and worsening asthma came after the hurricane and stacked on top of it, which explains why a family that coped at first is now overwhelmed. It beats D because adaptation (xX) is the outcome the family reaches, not the demands acting on it. Assessing pile-up keeps attention beyond the original event.

**B** **Incorrect** (trap: Wrong model or concept). The bB factor covers the family's existing and newly developed resources, such as savings, support and coping skills. The listed events drain resources; they are demands, not resources.

**C** **Incorrect** (trap: Wrong model or concept). The cC factor is the meaning the family gives to the original stressor, the later demands and its resources. The events themselves are not perceptions.

**D** **Incorrect** (trap: Wrong model or concept). The xX factor is the family's adaptation over time, ranging from good adjustment to maladaptation. Being overwhelmed reflects the outcome so far; the job loss, disputes and illness are inputs that shape it.

**Takeaway:** The Double ABCX pile-up factor captures stressors that accumulate after a crisis, explaining delayed family breakdown.

**Sources:** McCubbin, H. I., & Patterson, J. M. (1983). The family stress process: The Double ABCX model of adjustment and adaptation. Marriage & Family Review, 6(1-2), 7-37, aA factor: pile-up; Hill, R. (1949). Families Under Stress. Harper & Brothers, ABC-X model

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 109. INT-1510 (rev 1) · Best answer: A

**PRETEST** — unscored on this form; do not count it

An 82-year-old resident of an assisted living facility tells the visiting social worker that the monthly bill includes a charge for services the resident never received. The resident is cognitively intact and says, 'I hate making a fuss.' The worker has a good working relationship with the business office. What should the social worker do **FIRST**?

**A.** Coach the resident to raise the charge with the business office

**B.** Call the business office directly to dispute the charge for the resident

**C.** Contact the long-term care ombudsman about the billing error

**Why each option is right or wrong**

**A** **Correct.** Empowerment practice builds the client's own capacity to act and starts with self-advocacy before the worker steps in or escalates. The resident is cognitively intact and able to raise the issue; coaching addresses the reluctance ('I hate making a fuss') and leaves the resident better able to handle the next problem (Lee, 2001; NASW Code 1.02). It beats B because calling for the resident fixes this bill but does for a capable client what the client can do, which reinforces dependence.

**B** **Incorrect** (trap: Good but not best). Making the call is efficient and well-meant, but it takes over a task a capable resident can do and misses a chance to build confidence. Direct advocacy fits if the resident tries and is ignored, or asks the worker to act.

**C** **Incorrect** (trap: Right action, wrong time). The ombudsman is an appropriate outside channel if internal steps fail or the facility retaliates. Going there first skips the resident's own voice and the facility's internal process.

**Takeaway:** Empower capable clients to advocate for themselves first; the worker steps in, or escalates outside the agency, only if those efforts fail.

**Sources:** Lee, J. A. B. (2001). The Empowerment Approach to Social Work Practice: Building the Beloved Community (2nd ed.). Columbia University Press, empowerment practice: building client capacity; NASW Code of Ethics (2021), 1.02

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 110. ASM-1538 (rev 2) · Best answer: B

In the eighth session of a weekly group for 15-year-olds at a community center, members jokingly treat the social worker as a parent and compete for the worker's attention like siblings. They now share personal struggles openly and compare the group to their own households. Early sessions were marked by testing rules and challenging the worker. Which stage of group development is the group **MOST** likely in?

**A.** Power and control

**B.** Intimacy

**C.** Differentiation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Power and control is the earlier stage of testing rules, status struggles and challenges to the leader. The stem places that testing in the first sessions; the group has since moved into more personal, family-like relating.

**B** **Correct.** Garland, Jones and Kolodny's model, as presented by Hepworth et al. (2022), describes an intimacy stage in which the group develops a familial frame of reference: members relate to one another like siblings and to the leader like a parent, and personal sharing deepens. The parent-like treatment of the worker, sibling-style rivalry and open sharing fit this stage. It beats C because differentiation comes later, when members accept one another as distinct individuals and the group develops its own identity rather than a family template.

**C** **Incorrect** (trap: Wrong model or concept). Differentiation follows intimacy. Members then accept differences, the group has its own identity and internal frame of reference, and members rely less on the leader, unlike the parent-centered rivalry described here.

**Takeaway:** In the Garland, Jones and Kolodny model, the intimacy stage brings a familial frame: sibling rivalry, the leader seen as parent, deeper sharing.

**Sources:** Hepworth et al. (2022). Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, stages of group development (Garland, Jones & Kolodny model)

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 111. ETH-0104 (rev 2) · Best answer: D

A child welfare social worker has carried a caseload far above agency limits for a year. The social worker feels drained before each workday, has grown cynical about the families served and doubts that the work makes a difference. At night, the social worker lies awake thinking about overdue paperwork. Which condition is **MOST** likely?

**A.** Secondary traumatic stress
**B.** Vicarious trauma
**C.** Countertransference
**D.** Occupational burnout

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress is a trauma response to hearing about clients' traumatic experiences, with intrusive images, avoidance or hyperarousal. The sleeplessness here centers on overdue paperwork, not on reminders of clients' trauma, so the picture points to workload rather than trauma exposure.

**B** **Incorrect** (trap: Wrong model or concept). Vicarious trauma is a gradual shift in the worker's core beliefs about safety, trust and the world after long empathic engagement with trauma. The stem describes exhaustion and cynicism tied to caseload, not changed beliefs about danger or trust.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference is a worker's emotional reaction to a particular client, shaped by the worker's own history. Here the exhaustion and cynicism are general, tied to workload across all families rather than to one client.

**D** **Correct.** Burnout is a work-driven syndrome marked by emotional exhaustion, cynicism toward the people served and a reduced sense of accomplishment (Maslach et al., 2001). An overloaded caseload, feeling drained, cynicism about families and doubting one's impact match all three features. It beats A because secondary traumatic stress involves trauma-like symptoms, such as intrusive images of clients' experiences, from exposure to their trauma (Bride, 2007); this worker's nighttime worry is about paperwork. Both call for supervision and self-care, but the response to burnout centers on workload and the work environment.

**Takeaway:** Exhaustion, cynicism and low sense of accomplishment from workload signal burnout; trauma symptoms from clients' stories signal secondary traumatic stress.

**Sources:** Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, burnout dimensions; Bride, B. E. (2007). Prevalence of secondary traumatic stress among social workers. Social Work, 52(1), 63-70, whole article; McCann, I. L., & Pearlman, L. A. (1990). Vicarious traumatization: A framework for understanding the psychological effects of working with victims. Journal of Traumatic Stress, 3(1), 131-149, whole article

**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

### 112. INT-0004 (rev 1) · Best answer: B

**PRETEST** — unscored on this form; do not count it

In family sessions, whenever the parents try to set a curfew, their 14-year-old and one parent join forces against the other parent, and the discussion collapses. Which intervention is **MOST** consistent with structural family therapy?

**A.** Construct a three-generation genogram with the family
**B.** Coach the parents to decide the curfew together in session
**C.** Direct the parents to argue about the curfew each evening at home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A three-generation genogram is a Bowen technique for mapping multigenerational patterns. It can be informative, but it is assessment from a different model rather than a structural intervention on the coalition.

**B** **Correct.** Structural family therapy sees the problem as a structure in which a cross-generational coalition, here one parent allied with the adolescent, undercuts the parental subsystem. Coaching the parents to work out the curfew together in session, while blocking the adolescent from joining in, is an enactment with boundary making. It strengthens the parental hierarchy (Minuchin, 1974; Minuchin & Fishman, 1981). It beats A because a genogram belongs to Bowen family systems therapy, which tracks multigenerational patterns and differentiation rather than restructuring interactions in the room.

**C** **Incorrect** (trap: Wrong model or concept). Prescribing the symptom is a paradoxical directive from strategic family therapy, associated with Haley and the MRI group. It does not address the family structure that a structural therapist would target.

**Takeaway:** Structural family therapy uses enactment and boundary making to strengthen the parental subsystem and break cross-generational coalitions.

**Sources:** Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems, boundaries and coalitions; Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, enactment; boundary making

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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### 113. ETH-1606 (rev 1) · Best answer: D

A 68-year-old transgender woman on a hospital medical unit has stopped eating her meals and refuses physical therapy. She tells the social worker that staff keep calling her 'sir' and using her former name, which is printed on her wristband. She asks the social worker for help and agrees that her care team can be told. What should the social worker do **NEXT**?

**A.** Ask the patient to describe how the comments affect her
**B.** Request a psychiatric consult for her refusals and poor intake
**C.** Coach the patient to correct staff each time it happens
**D.** Ask the unit to update her name and pronouns and brief staff

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Exploring impact is often a good early step. Here the patient has already described the harm, tied it to her withdrawal and asked for help, so more exploration delays the change she requested.

**B** **Incorrect** (trap: Skips assessment). Refusals and poor intake deserve attention, and a psychiatric consult would fit if they persisted after the cause was addressed. Treating them first as a mental health problem ignores the clear cause she has named.

**C** **Incorrect** (trap: Good but not best). Self-advocacy skills can help, but asking a hospitalized patient to correct staff every time places the work of change on the person being harmed. The hospital is responsible for fixing its records and practices.

**D** **Correct.** Misgendering and use of a former name are microaggressions that signal a person's identity is not respected, and they can drive patients away from care (Nadal et al., 2012). The patient has named the harm, linked it to her withdrawal and agreed that the team may be told, so the next step is to fix the source: the record and staff practice. Holding institutions accountable is part of the social worker's role (NASW Code 1.05(c)). It beats C because coaching leaves the burden of repeated correction on the patient.

**Takeaway:** When staff misgender a patient, fix the source by correcting records and staff practice rather than leaving correction to the patient.

**Sources:** Nadal, K. L., Skolnik, A., & Wong, Y. (2012). Interpersonal and systemic microaggressions toward transgender people: Implications for counseling. Journal of LGBT Issues in Counseling, 6(1), 55-82, whole article; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 114. INT-1110 (rev 2) · Best answer: D

In a first session at a community mental health center, an adult client talks rapidly about eviction worries, a recent breakup, poor sleep, and conflict with a sibling, jumping between topics. Twenty minutes remain in the session. What is the **BEST** next move to begin setting priorities?

**A.** Focus the rest of the session on the eviction threat
**B.** Reflect how overwhelmed the client seems to feel right now
**C.** Ask closed questions to cover each problem in turn
**D.** Summarize the concerns and ask which one matters most

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Housing threats are serious, and the client may well choose this first. But the stem shows no emergency, and the worker deciding the focus alone skips the client's view of what is most pressing. A summary lets the client choose.

**B** **Incorrect** (trap: Good but not best). Reflecting the feeling of being overwhelmed is a sound empathic response and could come just before the summary. On its own, it does not organize the concerns or move toward priorities, which the remaining time calls for.

**C** **Incorrect** (trap: Misuses the technique). Closed questions gather facts but would turn the session into a checklist and use up the time without the client choosing a focus. They fit later, when a chosen problem needs specific detail.

**D** **Correct.** A summary ties scattered statements together, shows the client they were heard, and creates a natural transition to the next task (Hepworth et al.). With four concerns raised rapidly and limited time left, summarizing them and asking the client where to start moves toward shared priorities while keeping the client in charge of the agenda (NASW Code 1.02). It beats A because eviction may turn out to be the client's top concern, but the worker choosing it without asking takes the decision from the client and may miss what feels most urgent to them.

**Takeaway:** When a client raises many concerns at once, summarize them and let the client choose where to start; this shows listening and sets shared priorities.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following skills: summarizing responses; NASW Code of Ethics (2021), 1.02
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 115. ETH-1609 (rev 3) · Best answer: B

An autistic 26-year-old client at a community agency joins a job-search support group. After 2 sessions, the client tells the social worker that the bright lights and overlapping voices in the group room become overwhelming, so the client leaves early. The client still wants to be in the group. What is the **BEST** response by the social worker?

**A.** Coach the client to ask the group for lighting and noise changes
**B.** Arrange sensory changes, such as softer lighting and a break space
**C.** Offer individual sessions in place of the group

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Supporting self-advocacy is valuable, and the client may later choose to tell the group what helps. Making the client ask peers for changes, though, shifts the agency's duty to modify its own setting onto the client and requires a disclosure the client has not chosen.

**B** **Correct.** Strong reactions to sensory input are a recognized feature of autism (DSM-5-TR). Access means adjusting the environment so the client can take part, and agencies must make reasonable modifications when needed for equal access unless they would fundamentally alter the service (28 CFR 36.302(a)). The client still wants the group, and modest changes such as dimmer lights, turn-taking rules and a break space keep that choice. It beats A because coaching the client to request changes from peers puts the agency's duty to make reasonable modifications on the client and requires a disclosure the client has not chosen.

**C** **Incorrect** (trap: Good but not best). Individual sessions can meet some needs, but the client chose the group for its peer contact. Moving the client out instead of fixing the room offers a separate, lesser service when a reasonable change would allow full participation.

**Takeaway:** For neurodivergent clients, adjust the environment so they can join the service they chose, rather than asking them to endure or leave it.

**Sources:** ADA Title III regulations, 28 CFR 36.302(a); DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Autism Spectrum Disorder
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 116. ASM-1163 (rev 1) · Best answer: C

During a telehealth session, a client who takes phenelzine for depression says they ate aged cheese and dry salami at a party an hour ago. The client now has a pounding headache at the back of the head, a stiff neck, and a racing heart. What should the social worker do **FIRST**?

**A.** Contact the prescriber and wait for a call back
**B.** Have the client lie down and recheck the symptoms in an hour
**C.** Arrange immediate emergency treatment for the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The prescriber needs to know, and that call can follow. Waiting for a call back while blood pressure may be climbing risks a stroke or other serious harm.

**B** **Incorrect** (trap: Ignores risk). Rest suits an ordinary headache, but these symptoms after tyramine-rich food on an MAOI signal a possible hypertensive crisis. Waiting an hour could allow serious harm.

**C** **Correct.** Phenelzine is a monoamine oxidase inhibitor (MAOI). Eating foods high in tyramine, such as aged cheese and cured meats, while taking it can set off a hypertensive crisis, a dangerous spike in blood pressure marked by severe headache, stiff neck and palpitations (Kaplan & Sadock). This is a medical emergency, so the social worker helps the client get emergency care at once, confirming the client's location. It beats A because the prescriber should be informed, but waiting for a call back delays treatment that may prevent a stroke.

**Takeaway:** Severe headache, stiff neck or palpitations after tyramine-rich food on an MAOI signal hypertensive crisis: get emergency care now.

**Sources:** Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Monoamine oxidase inhibitors: hypertensive crisis and dietary restrictions
**Area** II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 117. ASM-1659 (rev 2) · Best answer: C

An emergency department social worker is asked to see a 68-year-old patient who was treated for a minor fall and is medically ready to leave. Transport home is arranged for 1 hour from now. The nurse notes that the patient seemed unsure how they would manage at home. What is the **BEST** way to approach the assessment?

**A.** Complete a full biopsychosocial history before the ride home arrives
**B.** Leave the assessment to the patient's primary care provider
**C.** Focus on home safety, supports and daily functioning now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). A full history is the ideal in ongoing care, but it is not realistic in an hour and is not needed for this decision. Spreading attention across every domain risks missing the home-safety facts that matter now.

**B** **Incorrect** (trap: Hands off too soon). Primary care can follow up, but the nurse's concern bears on today's discharge. Deferring the whole assessment sends an uncertain patient home without anyone checking supports or safety.

**C** **Correct.** A biopsychosocial assessment is tailored to the setting, the purpose and the time available, so its depth follows the decision at hand (Hepworth et al.). Here the decision is whether this patient can safely go home within the hour. The priority areas are home safety, available help, daily functioning and any risk. Less urgent areas, such as developmental or detailed family history, can be gathered later by community providers. It beats A because trying to cover every domain in an hour would thin out the very questions that decide a safe discharge.

**Takeaway:** Match the depth and focus of a biopsychosocial assessment to the setting, purpose and time; start with the domains that drive the immediate decision.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Multidimensional assessment: adapting scope to setting and purpose
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 118. ASM-1036 (rev 2) · Best answer: A

At a university diabetes clinic, a 20-year-old client with type 1 diabetes is at a normal weight but has rising blood sugar readings. The client describes eating large amounts of food in a short time about twice a week for 4 months, feeling unable to stop. Afterward, the client skips insulin doses 'to cancel out the calories.' The client says their weight decides how they feel about themselves. Which diagnosis is **MOST** likely?

**A.** Bulimia nervosa
**B.** Binge-eating disorder
**C.** Anorexia nervosa, binge-eating/purging type
**D.** Other specified feeding or eating disorder

**Why each option is right or wrong**

**A** **Correct.** Bulimia nervosa involves recurrent binges with loss of control plus recurrent inappropriate compensatory behavior, both at least weekly for 3 months, with self-worth unduly tied to shape or weight (DSM-5-TR). In people with diabetes, skipping or cutting insulin doses to undo calories is a recognized compensatory behavior (DSM-5-TR). It beats B because binge-eating disorder is diagnosed only when there is no regular compensatory behavior, and this client omits insulin after binges.

**B** **Incorrect** (trap: Near-miss diagnosis). Binge-eating disorder shares the binges and loss of control, but it excludes regular compensatory behavior. Deliberately skipping insulin to undo calories is compensation, which moves the diagnosis to bulimia nervosa.

**C** **Incorrect** (trap: Near-miss diagnosis). Anorexia nervosa requires significantly low body weight. This client is at a normal weight, so even with binge and compensation behavior, anorexia does not fit.

**D** **Incorrect** (trap: Near-miss diagnosis). This category covers serious eating problems that fall short of a full diagnosis, such as bulimia with binges and compensation less often than weekly or for under 3 months. This client binges about twice a week for 4 months and compensates afterward, so the full bulimia nervosa criteria are met (DSM-5-TR).

**Takeaway:** Insulin omission to undo calories is a compensatory behavior; with weekly binges and weight-based self-worth, it indicates bulimia nervosa.

**Sources:** DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Bulimia Nervosa
**Area** II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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### 119. INT-1631 (rev 1) · Best answer: C

Which case management model works mainly in the community and builds on neighbors, landlords and employers to reach goals the client chooses?

**A.** Broker model
**B.** Clinical case management
**C.** Strengths model
**D.** Assertive community treatment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Broker case management links clients to formal agencies and does little direct work with them. It neither centers the community as the work setting nor favors informal, natural resources over referrals to programs.

**B** **Incorrect** (trap: Wrong model or concept). Clinical case management combines therapy with resource coordination. The description involves no therapy; it centers on client-chosen goals pursued through informal community resources.

**C** **Correct.** Rapp and Goscha's (2012) strengths model holds that the community is an oasis of resources, the client directs the helping process, and the community rather than the office is the primary setting for work. Using landlords, neighbors and employers to reach client-chosen goals is its hallmark. It beats D because ACT also works in the community, but an ACT team mainly delivers treatment and support itself; the emphasis on naturally occurring resources ahead of formal programs marks the strengths model.

**D** **Incorrect** (trap: Wrong model or concept). ACT is community based too, which makes it tempting. But ACT is a multidisciplinary team that provides most services directly, around the clock, for people with frequent hospitalizations, rather than leaning first on neighbors and employers (SAMHSA ACT EBP KIT, 2008).

**Takeaway:** The strengths model of case management works in the community, follows client-chosen goals, and favors naturally occurring resources over formal services.

**Sources:** Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, Principles of the strengths model; SAMHSA (2008) Assertive Community Treatment Evidence-Based Practices KIT, Building Your Program: core features of ACT
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 120. ASM-1561 (rev 1) · Best answer: A

**PRETEST** — unscored on this form; do not count it

Engel's biopsychosocial model was proposed as a challenge to which approach to illness?

**A.** The biomedical model
**B.** The psychoanalytic model
**C.** The ecological model

**Why each option is right or wrong**

**A** **Correct.** Engel (1977) argued that the dominant biomedical model, which explains illness only through biological processes, could not account for how psychological and social factors shape illness, help-seeking and recovery. His biopsychosocial model became the frame for social work assessment across biological, psychological and social domains. It beats B because Engel's critique targeted medicine's reliance on biology alone, not psychoanalytic theory.

**B** **Incorrect** (trap: Wrong model or concept). Psychoanalytic theory explains distress through unconscious processes and was not Engel's target. His critique addressed medicine's practice of explaining illness through biological causes alone.

**C** **Incorrect** (trap: Wrong model or concept). The ecological perspective, which stresses the fit between person and environment, is compatible with Engel's model. It was not the approach his paper set out to challenge.

**Takeaway:** The biopsychosocial model holds that biological, psychological and social factors together shape illness, replacing biology-only explanations.

**Sources:** Engel, G. L. (1977). The need for a new medical model: A challenge for biomedicine. Science, 196(4286), 129-136, whole article
**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 121. ASM-1563 (rev 1) · Best answer: B

**PRETEST** — unscored on this form; do not count it

A court refers a client for assessment and describes the problem as 'anger and aggression.' At intake, the client says the real problem is losing visits with their children since a breakup and not knowing how to get them back. The social worker is writing the presenting problem section of the biopsychosocial assessment. What is the **BEST** way to record it?

**A.** Record the court's description as the official presenting problem
**B.** Record the client's own words along with the referral reason
**C.** Record the social worker's view of the underlying problem

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The court's concern belongs in the record as the referral reason, but treating it as the whole presenting problem leaves out how the client defines the difficulty, which is the starting point for engagement.

**B** **Correct.** The presenting problem is recorded in the client's own words, with the referral source's reason noted separately, because what a referral says and what a client identifies often differ (Hepworth et al., assessment of problems and strengths).

Recording both keeps the court's concern visible while starting where the client is: losing visits with their children. It beats A because recording only the court's label replaces the client's view of the problem and can weaken engagement from the first session.

**C** **Incorrect** (trap: Misuses the technique). Clinical interpretation belongs later, in the assessment summary or formulation. Placing it in the presenting problem section mixes the worker's hypotheses with the client's own account.

**Takeaway:** Record the presenting problem in the client's own words, and note the referral source's stated reason separately.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: problems and strengths (presenting problem)

**Area** II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 122. INT-1163 (rev 1) · Best answer: D

A high school student died by suicide over the weekend. The family has confirmed the cause of death and agreed that it may be shared. The principal asks the school social worker how to inform students on Monday morning. Which approach is **MOST** helpful?

**A.** Read a brief statement over the school intercom
**B.** Hold an assembly where staff share facts and resources
**C.** Say a student died and decline to discuss the cause
**D.** Have trained staff share the news in homeroom-size groups

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). An intercom announcement reaches everyone at once but leaves students alone with the news and gives staff no way to see reactions or answer questions. Postvention guidance advises against it.

**B** **Incorrect** (trap: Ignores risk). An assembly shares accurate facts, but in a large crowd staff cannot watch individual reactions, and the event can magnify the death. Guidance favors smaller settings for this reason.

**C** **Incorrect** (trap: Good but not best). Withholding a confirmed cause that the family agreed to share tends to fuel rumors and misses a chance to talk about suicide safely and point students to help. Discretion fits when a family asks for privacy.

**D** **Correct.** School postvention guidance recommends telling students in small groups, such as homerooms, where trained staff can share accurate facts, answer questions, and notice students who are struggling (American Foundation for Suicide Prevention & SPRC, 2011). Small groups support grief and help identify close friends and other vulnerable students for follow-up, which lowers contagion risk. It beats B because an assembly gives the same facts to a large crowd, where staff cannot watch individual reactions and the event can magnify the death.

**Takeaway:** After a student suicide, share facts in small groups led by trained staff, avoid assemblies, and identify vulnerable students for follow-up.

**Sources:** American Foundation for Suicide Prevention & Suicide Prevention Resource Center (2011). After a Suicide: A Toolkit for Schools, Crisis response: notifying students; identifying students at risk
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3